Initiating Coverage

16th March, 2023

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HCG Hospitals

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Healthcare Global Enterprise Ltd (HCG)

Initiating Coverage | **Hospitals**

At Inflection Point

DALAL & BROACHA
STOCK BROKING PVT. LTD.

Equity Research Desk

16th March 2023

Investment Argument:

HCG is a story of how a doctorpreneur created a world-class cancer hospital chain.HCG is a brainchild of an oncologist whose radical & multi-disciplinary approach towards cancer treatment combined with highest levels of research & technology in India is killing cancer affordably.Unlike other specialities oncology has to be treated right way the first time,where tumor is very notorious because there are chances of relapse.HCG is the largest oncology focused hospital in India with PAN India presence.

➢ Capex to Opex model to drive RoCE

Oncology/Cancer is a capex heavy business with **machines forming ~50% of total capex cost** (Average capex cost of Rs.500 Mn for a 60-80 beds will have ~Rs.250 Mn as machine cost).HCG has ~31 LINAC machines (vs 17 LINAC machines with Apollo Hospital).We expect RoCE to rise from 3% in FY20 to 13% in FY25E.***Refer Exhibit No.3,16,17,18 for more details***.

HCG has revamped its business model from **OWNING THE MACHINES TO PAY PER USE BASIS**.HCG is the only company in India who has been allowed machines on pay per use basis due to the sheer volume it generates in oncology which makes the unit economics viable for machine renting companies. Also,beyond certain volumes HCG does not pay anything to the machine renting company, as the volume increases the pay per patient also decreases. **(Out of the 31 LINAC machines 5 machines are on pay per use basis with 20% share in realization ; 5 additional pay per use machines are expected to be added in FY23 & 6 machines in FY24 which will significantly drive upward RoCE).**

➢ Margin expansion of 200-300 bps over next 2-3 years to improve EBITDA significantly

With emerging centers already performing well, the drag on margins will lower down and company is confident of 200-300 bps margin improvement with 100-150bps to be visible from Q1FY24 itself. Company has engaged one of the Big4 to start driving operational efficiencies at a one-time cost of Rs.50 Mn.

Financial Summary

Y/E Mar (Rs mn)	FY20	FY21	FY22	FY23E	FY24E	FY25E
Net sales	10,956	10,134	13,978	16,808	18,598	20,269
EBIDTA	1,722	1,266	2,380	3,023	3,501	3,871
Margins	15.7	12.5	17.0	18.0	18.8	19.1
PAT (adj)	-1,067	-1,144	-53	355	716	918
Growth (%)	306.2	76.2	-117.6	-46.8	174.7	35.5
EPS	-12.03	-15.43	3.87	2.55	5.15	6.60
P/E (x)	-22	-17	70	106	52	41
P/B (x)	6	5	4	4	4	4
EV/EBITDA (x)	17	29	16	13	11	10
RoE (%)	-28	-16	-1	4	7	9
ROCE (%)	3	-1	7	10	13	13
RoIC (%)	3	-1	4	8	11	12

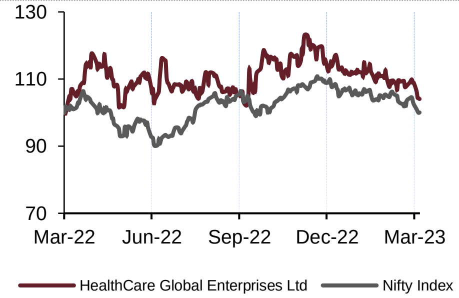
Source: Company, Dalal & Broacha Research

Rating	TP (Rs)	Up/Dn (%)
BUY	**391**	**45**

Market data		
Current price	**Rs**	**266**
Market Cap (Rs.Bn)	(Rs Bn)	37
Market Cap (US$ Mn)	(US$ Mn)	447
Face Value	Rs	10
52 Weeks High/Low	Rs	320 / 250
Average Daily Volume	('000)	47
BSE Code		539787
Bloomberg		HCG.IN

Source: Bloomberg

One Year Performance



Source: Bloomberg

% Shareholding	Dec-22	Jun-22
Promoters	71.38	71.39
Public	28.62	28.61
Total	**100**	**100**

Source: Bloomberg

Key Risks :

- ➢ Delayed payment from government schemes affecting working capital
- ➢ Dependency on single speciality & inability to scale
- ➢ Change of management/investors/professional management
- ➢ Slower-than-expected pace of break-even of new hospitals
- ➢ Slowdown in revenue intensity
- ➢ Execution risk in expansion projects
- ➢ Price capping by regulators
- ➢ Possibility of conflicts or disputes with existing doctor partners

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Key driver for EBITDA expansion would be scaling of emerging centers which are at ~9% EBITDA margin vs mature centers at ~25% EBITDA margin & company level EBITDA margin of ~21% (before allocation of corporate expenses)

➢ Largest oncology focused hospital with leadership in 13 out of 18 locations present

HCG has dominant network in cancer care with market leadership across 13 out of 18 cities in terms of revenue.HCG is India's largest oncology focused player with >1.6x footprint of comprehensive care centers as compared to next largest player. ***Refer Exhibit No.26 for more details***.

➢ Differentiated by technology,research & customer centric approach (tumor board)

HCG is a technology focused company & does specialized R&D on oncology. HCG has published close to 756 research papers till date. HCG follows a differentiated treatment approach for critical cases where all 400 oncologists connect through video conferencing & discuss critical cases by taking inputs from all doctors which they call it as the tumor board which is conducted every week. ***Refer Exhibit No.29 for more details***.

Also HCG has cutting edge technology in the form of cyberknife (2 machines) & ethos (1 machine used for adaptive radiotheraphy ; 1st in India to adapt that & now 2 other players have followed) which has become the USP for the company as these are very expensive machines ranging from Rs.250 Mn to 500Mn & available with very few hospitals across India. ***Refer Exhibit No.17,18 for more details***.

➢ Streamlining of operations to drive business excellence

HCG has been instrumental in connecting all the centers through various ERPs & softwares in last few years which has streamlined the processes & centre of excellence at Banglore becomes the sole decision maker w.r.t to any treatment thereby leveraging common infrastructure & reduce turn-around time. Also HCG has formulated a strategy team to drive business excellence leading to cost rationalisation & increased brand awareness.

➢ Oncology to outpace other speciailities in terms of growth

Oncology as a segment is expected to grow @ 12% & touch a market size of Rs.260Bn by FY24.Companies like HCG can outperform industry growth with their domain expertise in the oncology space along with PAN India presence & hub & spoke model. Out of the total ~3.7 lac new registrations in last 5 years close to 70-80k new registrations are with HCG (~3.7% of cancer care market by volume).New cancer incidences (I.e.Total active cancer patients) have increased from 12 lac patients in 2018 to

20L patients in 2022 (13.5% CAGR) due to increased awareness & diagnosis. Oncology is seeing a paradigm shift as day by day lifestyle related cancer cases are increasing drastically & thus growing at faster pace than other specialities.If we were to rank fastest growing specialities in India then cardiac would be no.1,oncology at no.2 & neurology at no.3. ***Refer Exhibit No.19 for more details***.

➢ Acquisition on table

Company has decided not to do any major capex going forward but focus on ramping up of existing facilities.However, for future growth HCG may look for acquisition based on operating cashflow it generates.HCG currently has a net debt position of Rs.2,110 Mn as of H1FY23 & is expected to generate close to ~Rs.2,500 Mn of operating cashflow this year.HCG aims to acquire oncology focused hospital in the EV/EBITDA range of 8-10x to expand its footprint.

➢ Turnaround in place

Company has delivered consistent revenue & EBITDA growth during last 7 quarters on YoY as well as sequential basis.Cost rationalization efforts with ramp up of new facilities remains a key driver for this turnaround.Going forward company will continue to focus on turning around its newer facilities which are at ~9% EBITDA margin vs mature facilities at 25%+ EBITDA margin (before allocation of corporate expenses). ***Refer Exhibit No.9 for more details***.

➢ Bed Capacity not a constraint in oncology ; Out-patients a key indicator in oncology business

HCG has ~2,000 commissioned beds & ~1,702 operational beds across 22 cancer care centers. Oncology has largely out-patients (2/3rd incase of HCG) & hence it is very easy to interchange beds into day care areas and vice-versa.Bed capacity is not a constraint incase of HCG. ***Refer Exhibit No.2 for more details***.

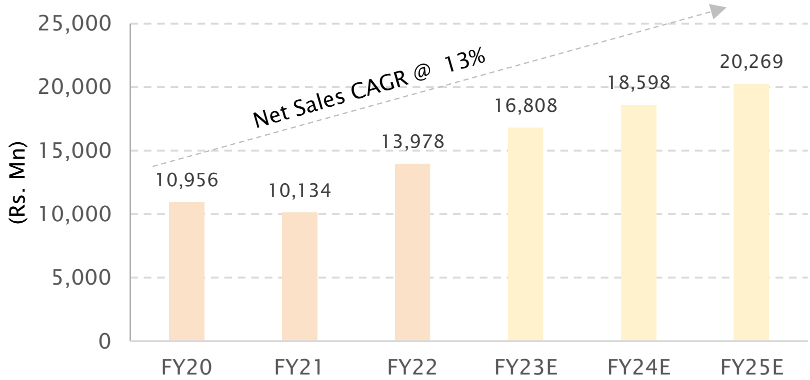
HCG receives 2/3rd of its revenue from out-patients which are a key driver in oncology business.It cannot be directly compared to multi-speciality hospital which has in-patients as key driver.More out-patients for HCG means better operating leverage.This number is likely to increase with increase in awareness & diagnosis.

➢ CVC Deal :

- HCG had heavy capex plans of ~Rs.10,000 Mn during FY16-20 & post that covid hit which led to leveraged balance sheet & poor operating performance
- At this point CVC (Luxembourg based private equity with US$133 billion of AUM) came in FY20 to save the company & bought a controlling stake.***Refer Exhibit No.27 for more details***
- Currently,they do not have plans to exit the company
- Dr.Ajaikumar plans to keep his stake at same level (~13% stake) right now & would not like to increase as he is at 71 years.

Source: Company, Dalal & Broacha Research

Exhibit 1: Sales to compund at 13% CAGR to outpace cancer industry growth of 12% CAGR



Source: Company, Dalal & Broacha Research

We expect sales CAGR of 13% over FY20-FY25E.Hereon, HCG's focus is to scale existing centers without doing incremental capex to drive RoCE.Focus will be on EBITDA & PAT for which company has already hired Big4 for driving operational excellence.On pricing front company is evaluating its peers but long term focus is to grow through volumes & relatively lower through price.

Historically HCG has compounded revenue at 18%/15%/13% on 10/5/3 year basis which has always been higher than the cancer industry growth & twice that of India's GDP growth.

Pricing dynamics & value proposition of HCG

HCG has pricing power when it comes to core metro markets where they have leadership position.Pricing is usually in the H1 category in metro markets of Bangalore,Ahemdabad etc.However,in places like Mumbai where HCG is a new entrant pricingwise they are competitive & usually 2-3rd in the list.HCG's value proposition is on treatment & not the infrastructure unlike various other hospitals.Whereas when it comes to Tier 2 cities HCG is premium on oncology services but noi premium on other allied services such as room rent.

Can sales grow without capex incase of hospitals?

Unlike other multispeciality hospitals,HCG is an oncology focused hospital with 2/3rd out-patients which do not require full-time beds.To increase out-patients one needs to focus on brand awareness which HCG is trying to leverage.Oncologists are also not a near term constraint in this business as HCG has the high density of oncologists at 2.5x of India's average indicating same doctors can handle more volumes.

Exhibit 2: Revenue Matrix

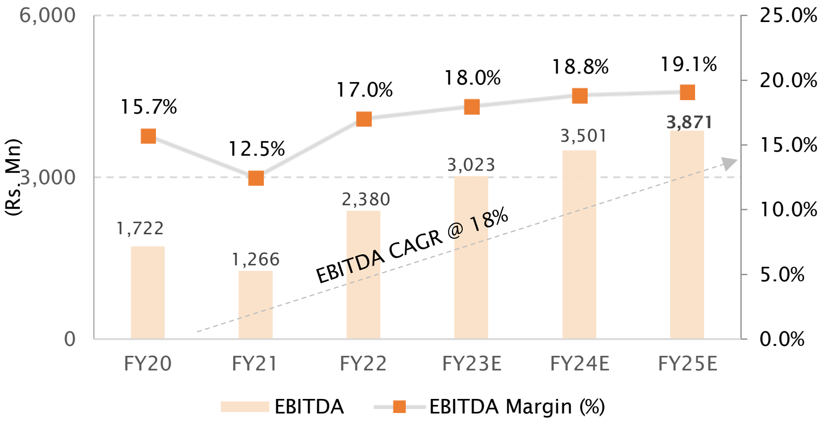
Particulars (Rs.Mn)	FY15	FY16	FY17	FY18	FY19	FY20	FY21	FY22	FY23E	FY24E	FY25E
Karnataka cluster	2,415	2,624	2,899	3,247	3,490	3,645	3,426	4,748	5,607	6,069	6,467
Growth Y-o-Y (in %)		9	10	12	7	4	-6	39	18	8	7
% of Onco	50	49	45	43	38	36	35	36	35	34	34
% of Total	46	45	41	39	36	33	34	34	33	33	32
Gujarat cluster	1,095	1,371	1,855	2,220	2,668	3,037	2,673	3,518	3,999	4,416	4,800
Growth Y-o-Y (in %)		25	35	20	20	14	-12	32	14	10	9
% of Onco	23	25	29	29	29	30	28	26	25	25	25
% of Total	21	23	26	27	27	28	26	25	24	24	24
East India cluster	318	414	491	587	642	826	872	1,168	1,604	1,786	1,959
Growth Y-o-Y (in %)		30	19	20	9	29	6	34	37	11	10
% of Onco	7	8	8	8	7	8	9	9	10	10	10
% of Total	6	7	7	7	7	8	9	8	10	10	10
Maharashtra cluster	-	193	244	672	1,239	1,390	1,607	2,275	2,412	2,702	2,938
Growth Y-o-Y (in %)	-	0	26	175	84	12	16	42	6	12	9
% of Onco	-	4	4	9	14	14	17	17	15	15	15
% of Total	-	3	3	8	13	13	16	16	14	15	14
Others	963	779	923	912	1,104	1,357	1,112	1,648	2,183	2,423	2,688
Growth Y-o-Y (in %)		-19	19	-1	21	23	-18	48	32	11	11
% of Onco	20	14	14	12	12	13	11	12	14	14	14
% of Total	19	13	13	11	11	12	11	12	13	13	13
Suchirayu Health	-	-	-	-	-	-	-	-	340	357	375
Oncology revenues	**4,791**	**5,381**	**6,412**	**7,638**	**9,143**	**10,255**	**9,690**	**13,357**	**16,145**	**17,752**	**19,226**
Growth Y-o-Y (in %)		12	19	19	20	12	-6	38	21	10	8
% of Total	92	92	92	92	93	94	96	96	96	95	95
Milann revenues	**403**	**461**	**590**	**669**	**644**	**701**	**444**	**621**	**663**	**846**	**1,042**
Growth Y-o-Y (in %)		14	28	13	-4	9	-37	40	7	28	23
% of Total	8	8	8	8	7	6	4	4	4	5	5
Total revenues	**5,194**	**5,842**	**7,002**	**8,307**	**9,787**	**10,956**	**10,134**	**13,978**	**16,808**	**18,598**	**20,269**
Growth Y-o-Y (in %)		12	20	19	18	12	-8	38	20	11	9

Total Oncology	FY15	FY16	FY17	FY18	FY19	FY20	FY21	FY22	FY23E	FY24E	FY25E
No of Centres	15	17	20	22	24	25	25	25	25	25	25
Beds (No.)	875	1,146	1,364	1,569	1,872	2,071	2,036	1,979	1,979	1,979	1,979
Operational beds (No.)	-	-	-	-	-	-	1,719	1,702	1,702	1,702	1,702
Occupied beds days (No.)	1,93,949	2,01,513	2,20,158	2,53,738	2,98,593	3,24,288	3,03,679	3,62,177	3,86,363	4,00,259	4,14,622
Avg Occupancy rate (in %)	60.7	51.0	46.9	44.5	43.7	42.9	48.4	58.3	62.2	64.4	66.7
ALOS (Days)	3.00	2.93	2.86	2.39	2.25	2.27	2.29	2.28	-	-	-
ARPOB (Rs.)	24,647	26,700	29,122	30,832	31,423	32,767	32,632	36,697	41,787	44,351	46,371
Growth Y-o-Y (in %)	12.8	8.3	9.1	5.9	1.9	4.3	-0.4	12.5	13.9	6.1	4.6
Oncology revenues (Rs.Mn)	4,791	5,381	6,412	7,638	9,143	10,255	9,690	13,357	16,145	17,752	19,226
Growth Y-o-Y (in %)	0.0	12.3	19.2	19.1	19.7	12.2	-5.5	37.8	20.9	10.0	8.3

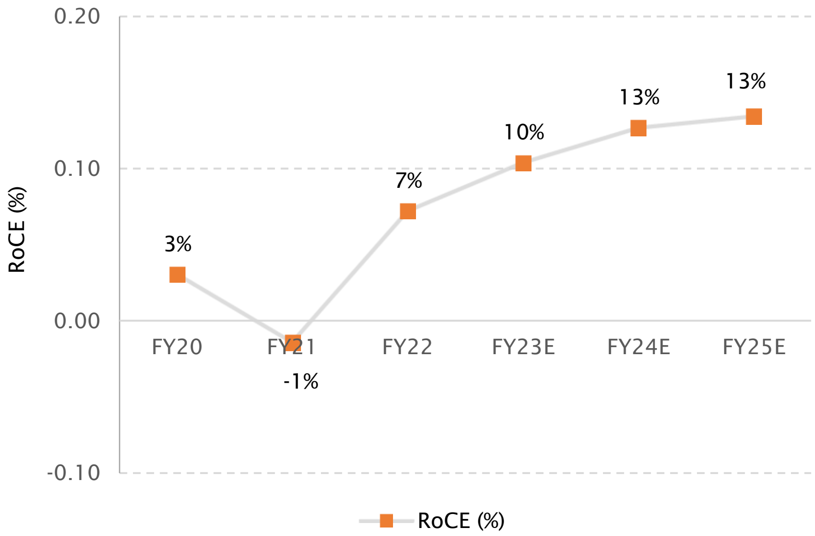
Source : Company,Dalal & Broacha Research

Major driver of revenue incase of HCG is the cancer care centre which contributes ~96% of overall revenue whereas Milann which is the IVF segment is ~4% of overall revenue.Major revenue growth levers are scaling of emerging centers & price rationalisation at existing centers.HCG is actively looking for acquisition based on operating cashflow it generates to drive further revenue growth.HCG currently has a net debt position of Rs.2,110 Mn as of 30th September 2022 & is expected to generate close to ~Rs.2,500 Mn of operating cashflow this year.HCG aims to acquire oncology focused hospital in the EV/EBITDA range of 8-10x to expand its footprint.Also to ensure consistent volumes from newly acquired hospital,HCG tries to maintain limited share of existing doctor so that he has his skin in the game.

Exhibit 3: Focus on EBITDA & lower capex to drive RoCE



Particulars	FY18	FY19	FY20	FY21	FY22	H1FY23	FY23E	FY24E	FY25E
Capex (Rs. Mn)	2,873	2,115	1,265	354	704	629	1,000	1,000	1,500
Capex (% Of Revenue)	35%	22%	12%	3%	5%	8%	6%	5%	7%



Source : Company,Dalal & Broacha Research

With emerging centers already performing well, the drag on margins will lower down and company is confident of 200-300 bps margin improvement with 100-150bps to be visible from Q1FY24 itself.Company has engaged one of the Big4 to start driving operational efficiencies at a one-time cost of Rs.50 Mn.Key driver for EBITDA expansion would be scaling of emerging centers which are at ~9% EBITDA margin vs mature centers at ~25% EBITDA margin & company level EBITDA margin of ~21% (before allocation of corporate expenses)

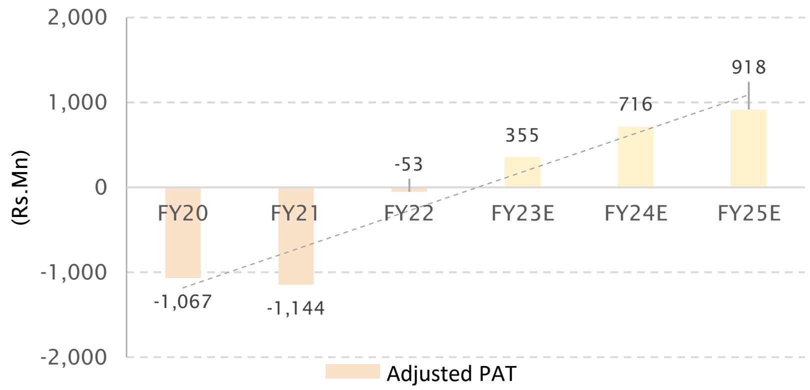
Ever wondered at what level does operating leverage start to kick in?

Stage 1 : For a new facility which is started,one needs to staff atleast 100 people assuming an occupancy level of 25-30% (Staff,medical equipments,consumables etc).Facilities which are in these range may not contribute significantly to overall EBITDA.

Stage 2 : One does not need to proportionately increase staff for 40-45% occupancy level as dynamic manpower such as clinical staff,nursing & paramedic do not need to be increased proportionately & that brings in operating leverage.They will start contributing to EBITDA gradually.

Stage 3 : Also as the asset turnover reaches 0.6-0.8x,assets start sweating more & operating leverage kicks in as there is no incremental machinery cost & staff expenses also don't increase proportionately beyond a point.This will have major impact on EBITDA.It usually takes 2-3 years after a new hospital is incorporated.

Exhibit 4: PAT to turn positive in FY23 indicating turnaround inplace ; 9M PAT : Rs.+321 Mn



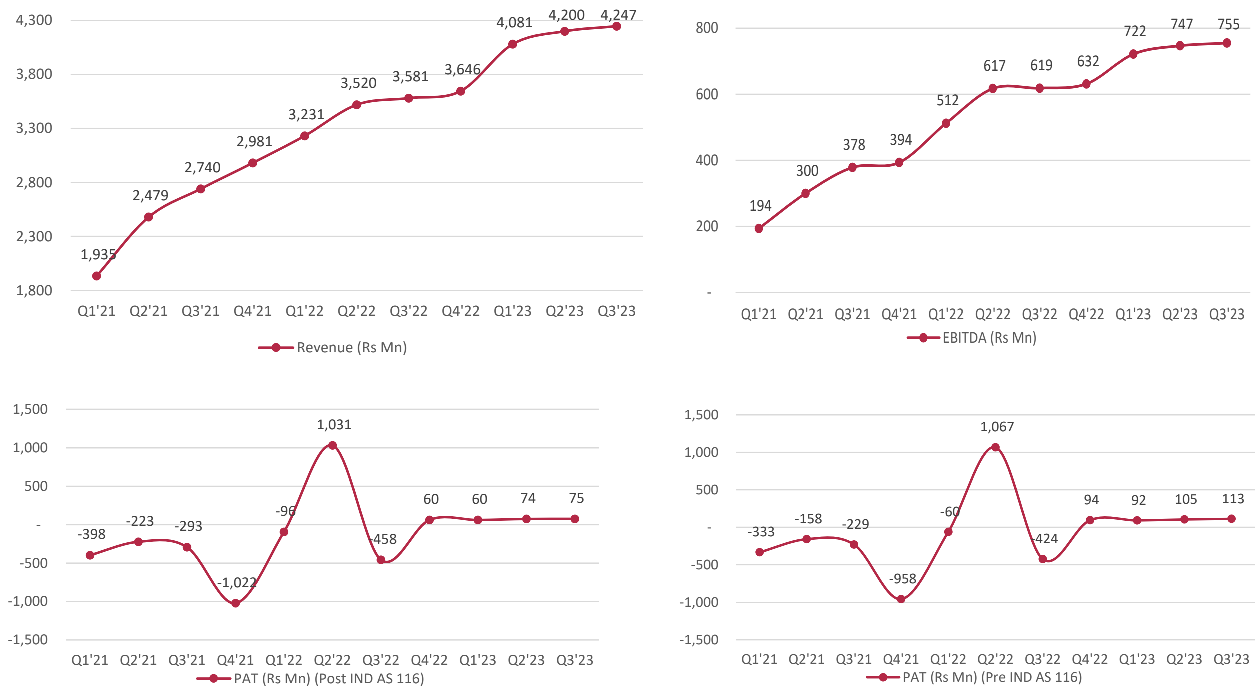
Source : Company,Dalal & Broacha Research

Over last few years PAT loss was on account of accelerated depreciation (non-cash in nature) & heavy interest burden due to heavily leveraged balance sheet.But with CVC capital infusion interest burden has reduced significantly & with low incremental capex,depreciation intensity is also likely to go lower turning PAT into positive.Other factors such as turning around of emerging centers,driving operational excellence,creation of digital footprint & brand awareness along with operating leverage are key contributors to PAT.

Is PAT a correct matrix to track company?

PAT has various non-cash items & book entries which may show a true picture about the company.So ideally one should look at CFO & FCFF to gain true understanding & valuation of the company.Aditionally,one can even look at CFO/EBITDA conversion to understand the conversion of cashflow at EBITDA level.

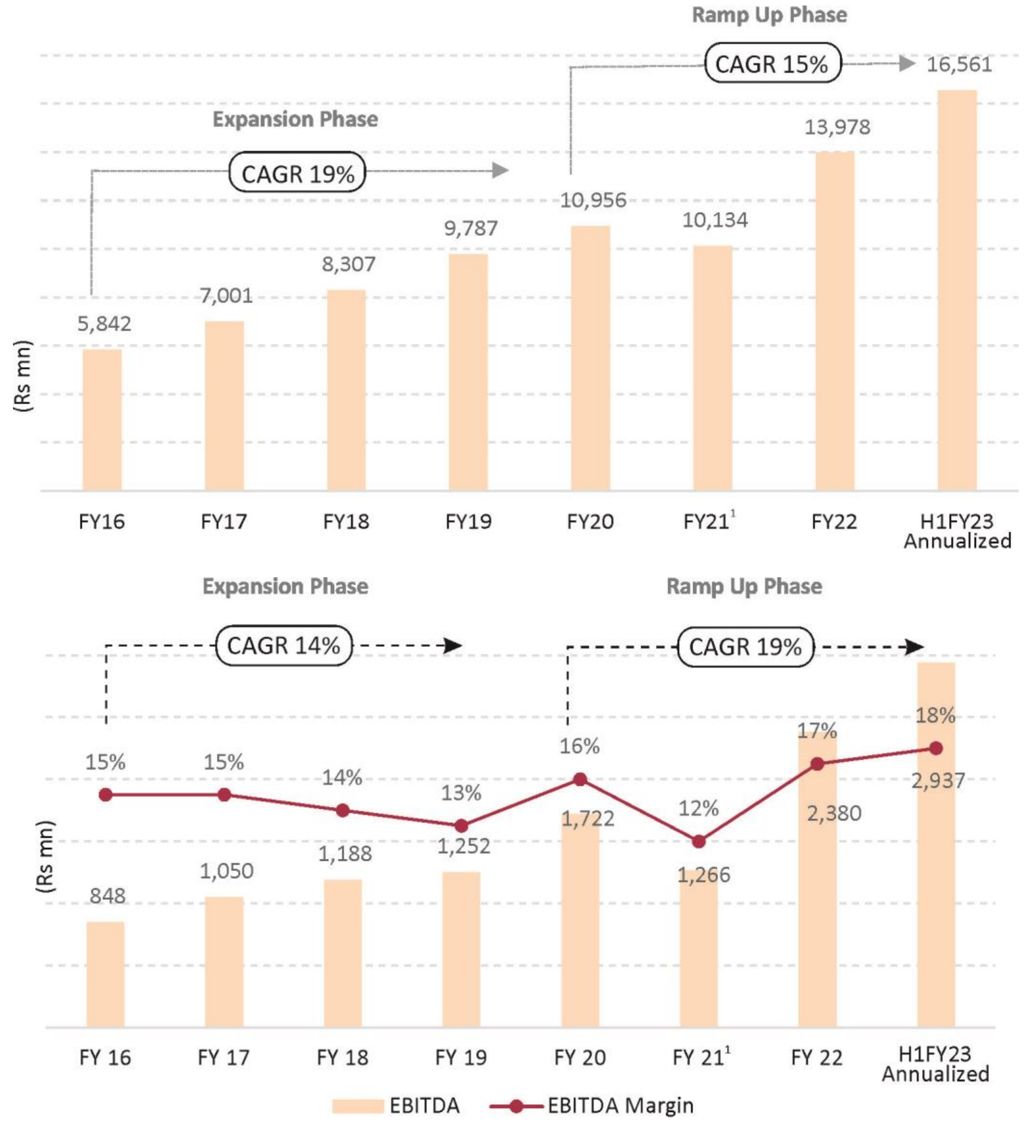
Exhibit 5: Runway to profitability ; Turnaround in play



Source : Company,Dalal & Broacha Research

HCG has continued its streak of highest ever revenue of Rs.4,247 Mn for the 8th quarter (Q3FY23) in a row & highest ever EBITDA of Rs.755 Mn (Q3FY23) for 7 quarters in a row.Over last few quarters,HCG has enhanced its clinical service portfolio,increased its clinical bandwith,go-to-market initiatives to increase its reach through online & offline presence.

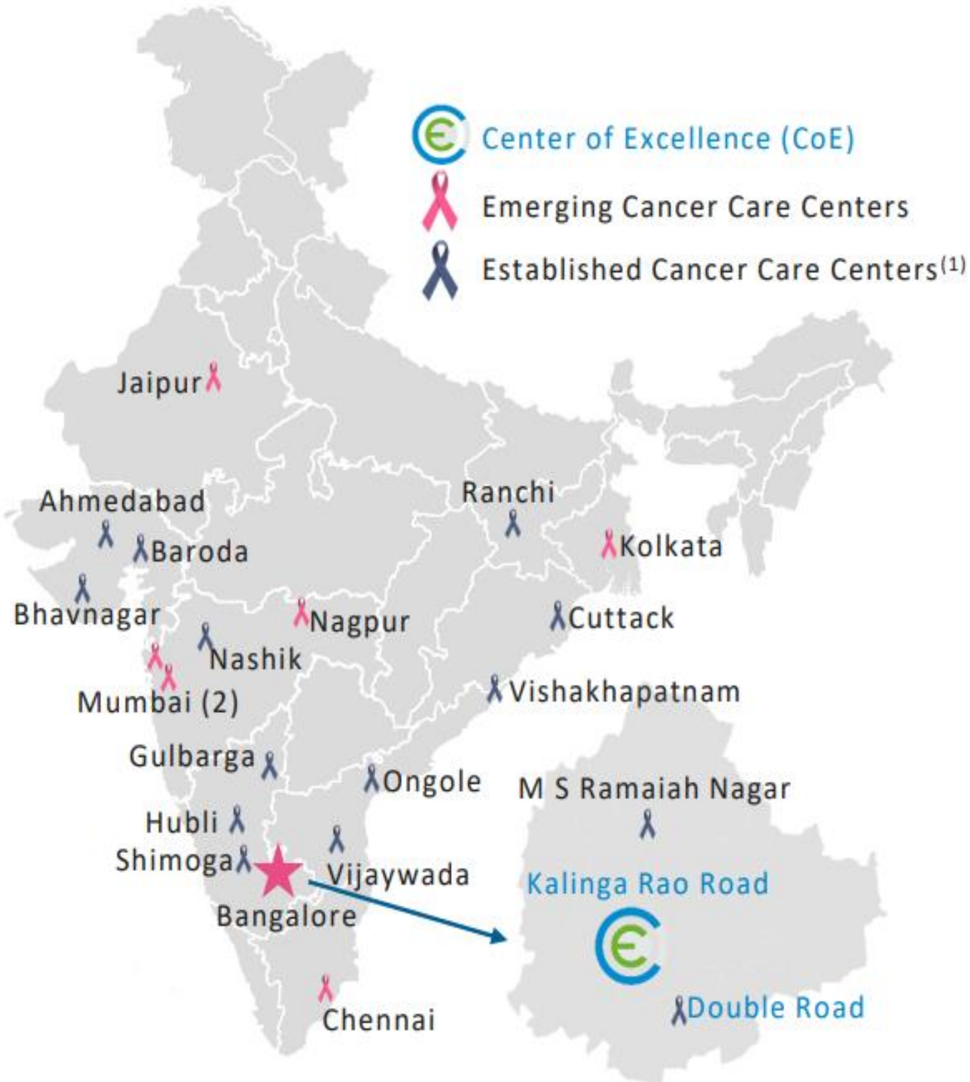
Exhibit 6: Expansion & Ramp up phase



Source : Company,Dalal & Broacha Research,1.Impacted due to Covid-19 headwinds

Usually during expansion phase,there is compromise on EBITDA as scaling of revenue is the focus area,whereas in the ramp-up phase there is a bump-up in EBITDA as scale benefits start accruing & due to enhanced brand visibility & revenue growth operating leverage kicks in.This is true incase of HCG hospital as well.Hospitals are valued on EV/EBITDA & higher EBITDA growth coupled with positive cashflow EV/EBITDA multiple starts showing upward trajectory. **Know more about valuation at the end of report.**

Exhibit 7: HCG has the largest cancer care network of 21 centers in India



Source: Company, Dalal & Broacha Research,

1. Established centers were operational before 2017

Exhibit 8: Cluster Breakdown of HCG

Location of the comprehensive cancer center	Commencement of Operation (calendar year)	Maturity	Leadership Status	Market Share (%)	Centre Type	Number of Operational Beds (Includes ICU Beds)	Number of RT-LINACs	Number of Operation theatres
Karnataka Cluster								
Bengaluru - Double Road	1989	EstC	Leader	~31%	CCC	48	1	3
Shimoga	2003	EstC	Leader		CCC	47	1	3
Bengaluru - Kalinga Rao Road (Center Of Excellence)	2006	EstC	Leader	~31%	CCC	194	3	7
Bengaluru - MS Ramaiah Nagar	2007	EstC	Leader		CCC	33	1	1
Hubli	2008	EstC	Leader		CCC	31	1	2
Gulbarga	2016	EstC	Leader		CCC	43	1	3
Hubli (Suchirayu Hospitals)	2021 (Acquired)	EmerC	Leader		MS	118	0	No Data
					Total	**514**	**8**	**19**
Gujarat Cluster								
HCG Hospitals Rajkot	2007	EstC	-		MS	60		No Data
HCG Hospital Ahmedabad	2007	EstC	Leader		MS	103	-	No Data
HCG Cancer Care Ahmedabad	2012	EstC	Leader		CCC	90	2	5
Baroda	2016	EstC	Leader		CCC, MS	63	1	3
Bhavnagar	2018	EstC	Leader		CCC, MS	87	1	3
					Total	**403**	**4**	**11**
East India Cluster								
Ranchi	2008	EstC	Leader		CCC	74	1	3
Cuttack	2008	EstC	Leader		CCC	116	2	3
Kolkata	2019	EmerC	Not a leader		CCC	49	1	2
					Total	**239**	**4**	**8**
Maharashtra Cluster								
Nashik Phase I	2007	EstC	Leader		CCC	95	1	2
Borivali	2017	EmerC	Not a leader		CCC	63	1	5
Nagpur	2017	EmerC	Leader		CCC	63	1	4
South Mumbai	2019	EmerC	Not a leader		CCC	25	2	2
Nashik Phase II	2018	EmerC	Leader		CCC	75	2	5
					Total	**321**	**7**	**18**
Andhra Pradesh Cluster								
Vijaywada	2009	EstC	Leader		CCC	75~	3	4
Ongole	2012	EstC	Leader		CCC	30^	1	2``
Vishakhapatnam	2016	EstC	Top 3		CCC	50	1	2
					Total	**155**	**5**	**8**
Others								
Chennai	2012	EmerC	Not a leader		CCC	5	1	-
Jaipur	2018	EmerC	Top 3		CCC	65	2	2
Kenya	2018	EmerC	Not a leader		CCC	-	-	-
					Total	**70**	**3**	**2**
					Total	**1702**	**31**	**66**

Summary of Total Hospitals	**No.**
Multispeciality Hospitals	3
Comprehensive Cancer Care Centres (Including Kenya & Multispeciality Bhavnagar)	22
Total Hospitals	**25**

Source: Company,,Dalal & Broacha Research

EstC = Established Centre ; EmerC = Emergency Centre

CCC = Comprehensive Cancer Care Centres ; MS = Multi-specialty hospitals

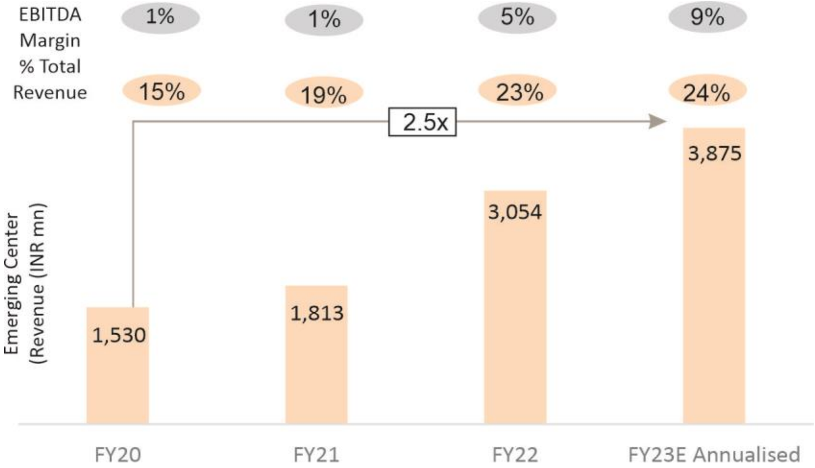
** HCG uses PET-CT Scanners of their partner*

~ Additionally, HCG has 120 self-care beds in their Vijayawada facility

^ Additionally, HCG has 61 self-care beds at Ongole

`` Includes a WBRRS System and Cyber Knife

Exhibit 9: Established/Mature centers vs emerging centers



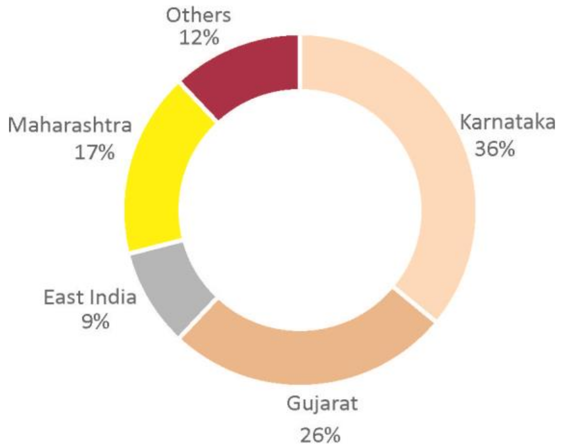
Particulars	FY20	FY21	FY22	FY23E Annualised	CAGR FY20-23E
Revenue from Emerging Centres^ (Rs.Mn)	**1,528**	**1,812**	**3,052**	**3,875**	**-**
Growth (%)		19	68	27	36
% of Total Revenue	15	19	23	24	-
EBITDA from Emerging Centres^* (Rs.Mn)	17	21	154	348	-
EBITDA Margin (%)	**1.1**	**1.2**	**5.0**	**9.0**	**-**
Revenue from Established Centres (Rs.Mn)	**8,727**	**7,878**	**10,305**	**12,270**	-
Growth (%)		-10	31	19	12
% of Total Revenue	85	81	77	76	0
EBITDA from Established Centres* (Rs.Mn)	1,501	1,675	2,581	3,086	-
EBITDA Margin (%)	**17.2**	**21.3**	**25.0**	**25.2**	**-**
Total Revenue (Rs.Mn)	**10,255**	**9,690**	**13,357**	**16,145**	
Growth (%)		-6	38	21	16
Total EBITDA* (Rs.Mn)	1,518	1,696	2,735	3,434	-
EBITDA Margin (%)	**14.8**	**17.5**	**20.5**	**21.3**	**-**

Source: Company, Dalal & Broacha Research

Note : ^Emerging Center represents centers operational after 2017,*Excl. Corporate Expenses,Certain assumptions have been made while for comparative purpose

~2/3rd of HCG revenue comes from established centers whereas 1/3rd of revenue comes from emerging centers. With emerging centers already performing well, the drag on margins will lower down and company is confident of 200-300 bps margin improvement with 100-150bps to be visible from Q1FY24 itself.Company has engaged one of the Big4 to start driving operational efficiencies at a one-time cost of Rs.50 Mn.Key driver for EBITDA expansion would be scaling of emerging centers which are at ~9% EBITDA margin vs mature centers at ~25% EBITDA margin & company level EBITDA margin of ~21% (before allocation of corporate expenses)

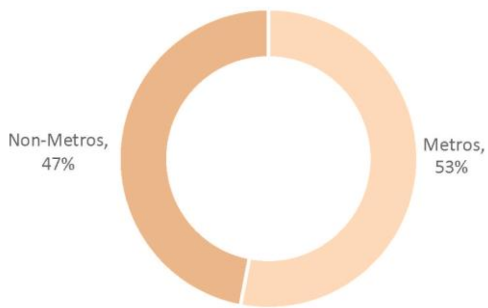
Exhibit 10: HCG has diversified geographical presence across India thereby deleveraging geographical risk



Source: Company, Dalal & Broacha Research,Data pertains to H1FY23 & does not include Kenya

Exhibit 11: Revenue by city type ; HCG Revenue Split in Metros & Non-Metros

City Type	No.of Centres In India	H1FY23 Revenue (Rs.Mn)	H1FY23 Revenue (%)	Industry Volume Growth Rate(%)	Industry Realisation Growth Rate(%)	Industry Growth Rate(%)
Metros	8	4,389	53%	8%	4%	12%
Non-Metros	13	3,892	47%	12%	3%	15%
Total	**21**	**8,281**	**100%**			**12%**



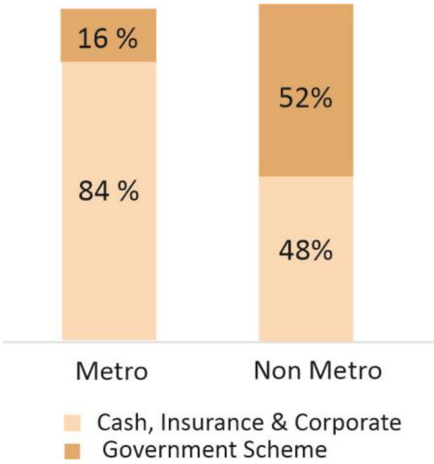
Key Parameters(1)	Metro	Non-Metro
Revenue CAGR(2)	18%	17%
Average EBITDA %	25%	23%
Payor Mix(3)	84%	52%

Source: Company, Dalal & Broacha Research,Metro & Non-metro data pertains to H1FY23

1. *For established centers only*
2. *CAGR is for the period FY20 to H1FY23*
3. *Payor Mix for cash, insurance and corporate, excluding government scheme related patients*

Non-metros are expected are to grow at faster pace due to under-penetration in non-metros.HCG focuses on non-metros which is expected to grow at 15% vs metros which is expected to grow at 12% to outpace cancer industry growth of 12%.HCG's strategy in non-metros is to focus on volumes by doing tie-up with various government schemes.In the initial phase there maybe a drag on margins but with scale benefits & brand awareness it can gradually expand its margins.Pricing is dynamic in different regions based on competitive intensity & its leadership position in different regions.

Exhibit 12: Healthy Payer Mix to reduce business risk

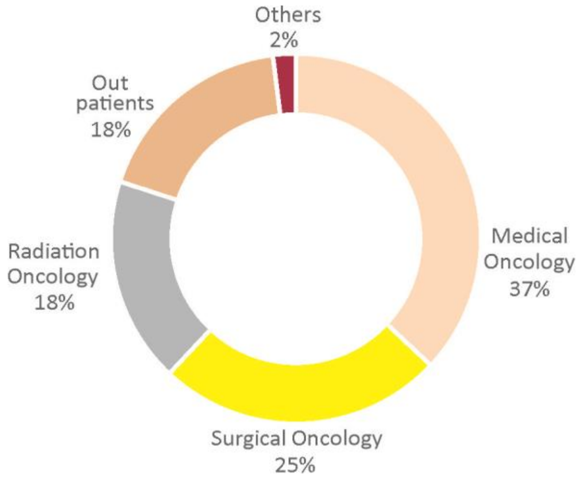


Source: Company, Dalal & Broacha Research

In metro cities close to 84% patients are through cash, insurance and corporate (excluding government scheme related patients) whereas in non-metros they are close to 52% for H1FY23.The reason of higher government patients in non-metros is to generate volume & create brand awareness.Once the centre turns mature payer mix gradually tweaks towards cash & insurance patients from government schemes.

Ever wondered why a corporate would do tie-up with standalone cancer care hospital vs multi-speciality?
Corporates usually include navratnas,public sector enterprises & private companies.They usually do tie-up with all hospitals to provide choice to their employees.They may do a tie-up with HCG as well as Apollo.In order to offer best of services to their employees they do tie-up with multiple hospital & leave upto the patient to decide whichever hospital they want to pick & choose from

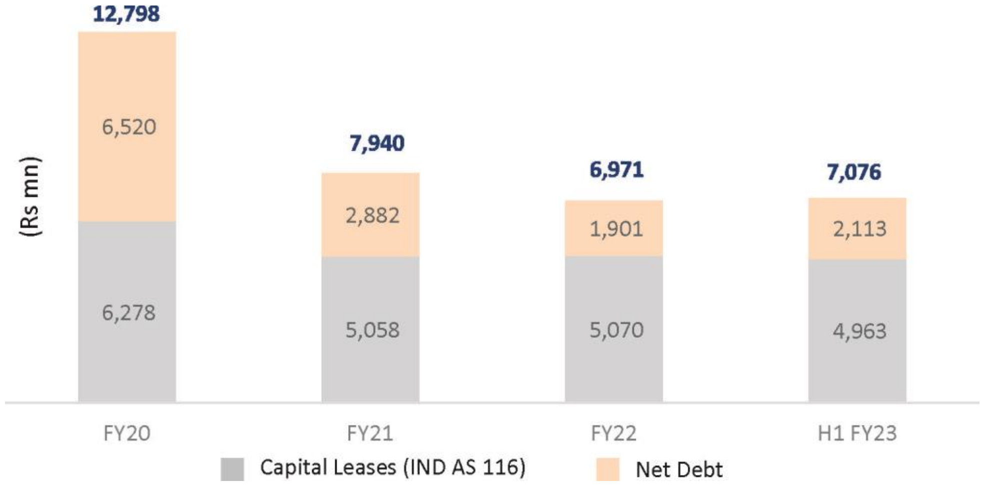
Exhibit 13: HCG has a diversified revenue from various modalities



Source: Company, Dalal & Broacha Research

Diverse modalities help in reducing revenue risk as company is not dependent on specific modality.

Exhibit 14: HCG has significantly deleveraged itself ; debt reduction underway

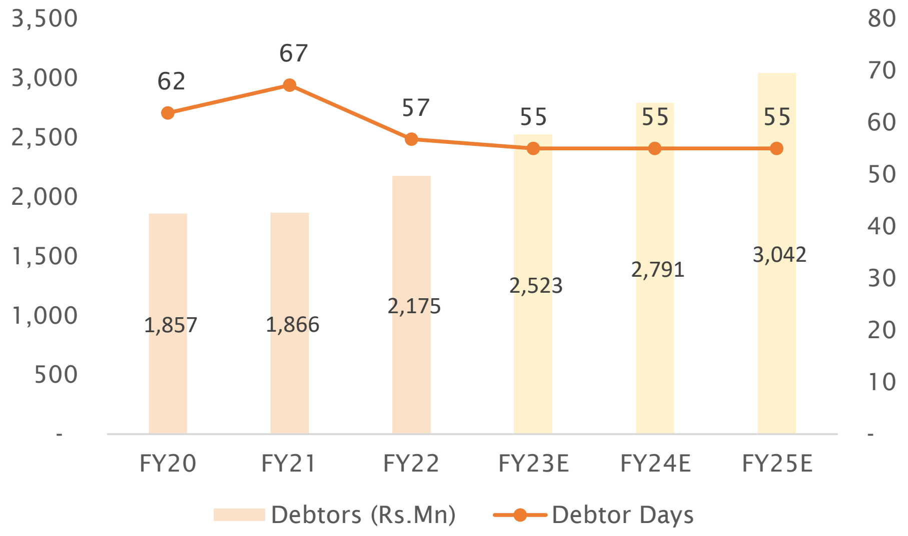


Particulars (Rs. Mn)	FY20	FY21	FY22	H1 FY23
Capital Leases	6,278	5,058	5,070	4,963
Net Debt	6,520	2,882	1,901	2,113
Total Debt including Capital Leases	**12,798**	**7,940**	**6,971**	**7,076**

Source: Company, Dalal & Broacha Research

1. *Significant deleveraging in the past few years*
2. *Comfortable debt position with well capitalized balance sheet*

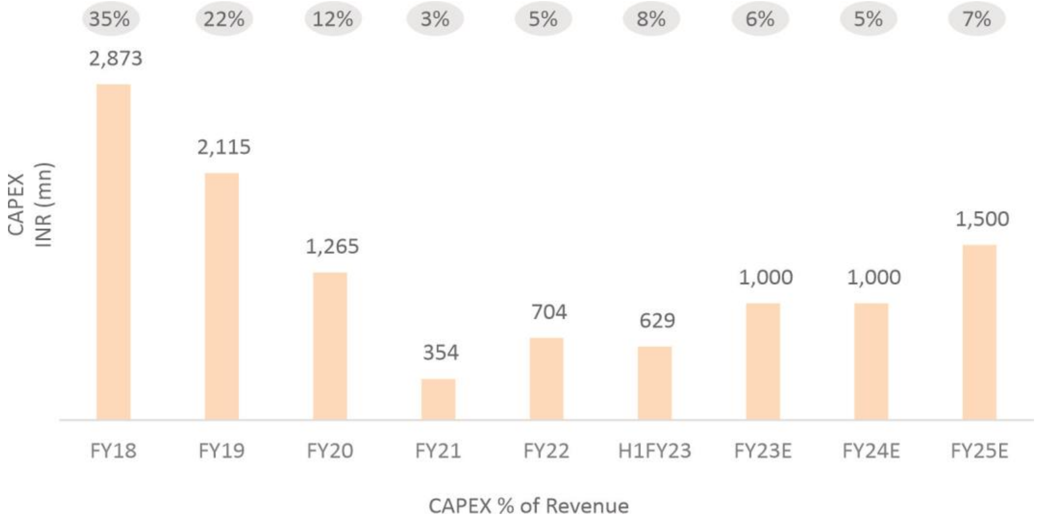
Exhibit 15: Debtors O/s inline with industry



Source: Company, Dalal & Broacha Research

HCG debtor days are inline with industry or sometimes even better.Every quarter HCG monitors its debtors closely with peers in industry.Outstanding debtors are expected to go down significantly as more & more digitalization is happening & processes are becoming streamlined.

Exhibit 16: Reduced intensity of capex to drive RoCE

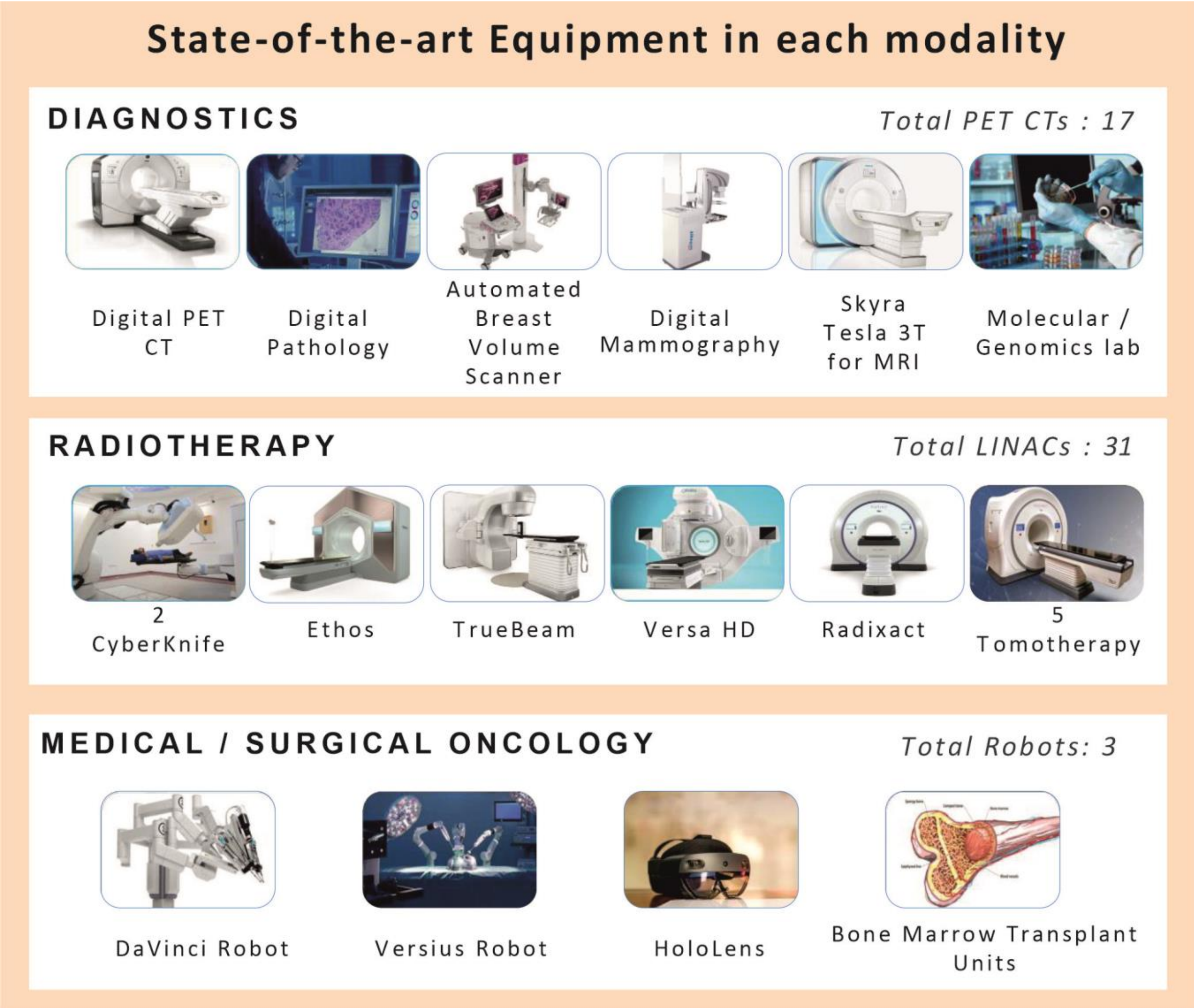


Source: Company, Dalal & Broacha Research

HCG has low future capex requirement.Capex is being planned at Ahemdabad – Phase II & whitefield capex at Bangalore – Center of Excellence.Besides these they are no major capex plans.

Exhibit 17: Medical equipments used at HCG hospitals

Sr.No.	Product Types	Product Category	No. of Machines	No.of Additional Machines Expected	Equipments
1	PET CTs	Diagnostics	17	-	Digital PET CT, Molecular / Genomics lab
2	LINACs	Radiotherapy	31	5	Cyberknife (2), Ethos, Tomotherapy (5)
3	Robots	Medical/ Surgical Oncology	3	-	DaVinci Robot, Bone Marrow Transplant Units
		Total	**51**	**-**	



Source: Company Investor Presentation, Dalal & Broacha Research, Refer FAQs to understand more on terminologies

Unique Pay per use – Game changer

HCG has revamped its business model from **OWNING THE MACHINES TO PAY PER USE BASIS**.HCG is the only company in India who has been allowed machines on pay per use basis due to the sheer volume it generates in oncology which makes the unit economics viable for machine renting companies.Also,beyond certain volumes HCG does not pay anything to the machine renting company, as the volume increases the pay per patient also decreases. **(Out of the 31 LINAC machines 5 machines are on pay per use basis with 20% share in realization ; 5 additional pay per use machines are expected to be added in FY23 & 6 machines in FY24 which will significantly drive upward RoCE).**

HCG has a unique arrangement with Elekta (Elekta is a global swedish company that develops and produces radiation therapy and radiosurgery-related equipment) to use machines on **PAY PER USE** basis.This unique business model could be game changer as it reduces capex cost significantly thereby increasing RoCE & growth.Also HCG is the only company who has been granted this permission to use machines on **PAY PER USE** basis.

Oncology/Cancer is a capex heavy business with **machines forming ~50% of total capex cost** (Average capex cost of Rs.500 Mn for a 60-80 beds will have will have ~Rs.250 Mn as machine cost).HCG has ~31 LINAC machines (vs 17 LINAC machines with Apollo Hospital).We expect RoCE to rise from 3% in FY20 to 13% in FY25E ***Refer Exhibit for more details.***

To know more about Elekta & its machine refer attached link:
https://www.elekta.com/products/radiation-therapy/

Exhibit 18: Quick glance at various machines/equipments used at HCG

Type of Equipment	Number	Uses
Other Linear Accelerators	16	A linear accelerator is a machine used in radiation treatment. It uses microwave technology to accelerate electrons in a part of the accelerator called the "wave guide". These electrons then collide with a heavy metal target to produce high energy x-rays. These high energy x-ray beams exit the machine from to conform to the shape of the patients tumor and exit through a part of the machine called the gantry. These beams destory the cancer cells without affecting the surrounding tissues. Radiation can be delivered to the tumor from many angles by rotating the gantry and moving the treatment couch.
Cyberknife	2	The cyberknife is the only robotic radiosurgery system that features a linear accelerator (LINAC) directly mounted on a robot to deliver the high-energy x-rays or photons used in radiation therapy. The robot moves and bends around the patient, to deliver radiation doses from potentially thousands of unique beam angles
Ethos	1	Uses AI to accurately map out the anatomy, target the location of the tumor, and optimize treatment plans in real time.
True Beam	1	TrueBeam delivers a prescribed radiation dose from nearly any angle. It combines imaging and beam delivery with technology that rotates around patients to accurately and precisely target tumors with great speed
Versa HD	5	Comes with 3D and 4D imaging feature, which helps the radiologists visualise very small tumours and their movements, useful in treatment of stereotactic radiotherapy (SRT) and stereotactic radiosurgery (SRS)
Tomotherapy	5	Tomotherapy treatment combines treatment planning, CT image-guided patient positioning and treatment delivery into one integrated system.
Radixact	1	A next-generation tomotherapy machine, designed to treat both benign and malignant tumours with superior precision and positively impact the clinical outcomes
Total	**31**	

Source: Company, Dalal & Broacha Research

HCG has 31 LINAC machines which are used in radiation therapy vs Apollo which has 17 LINAC machines.HCG has been at the forefront in bringing newer & affordable technology to India.

Apollo's Proton therapy vs HCG's Ethos

Proton therapy (used by Apollo Hospitals) is for limited cancer incidences such as pediatric & prostate cancer.Challenge is that incremental benefit coming out of proton therapy compared to any other modality of treatment within those limited cancer incidences is not significant enough to justify the investment it takes.There are newer techniques which have evolved.There is no clear cut published data which endorses the outcome of proton therapy is far superior to any other machine.Cyberknife of HCG also treats prostate cancer.with incremental benefit being very high as the same machine can be used to treat several other types of cancer.HCG focuses on equipments which are multi-purpose in nature (Ethos at centre of excellence which is an adaptive radiotherapy machine & incremental benefit is higher since it can be used for multiple therapies including pediatric & prostate).

Proton therapy requires an initial investment of Rs.2,000-2,500 Mn vs Ethos requiring an initial investment of Rs.350-500 Mn.Also because of the limited use proton unit economics are not that great compared to Cyberknife/Ethos thereby generating better RoCE for the company without compromising on the quality of treatment.Also proton therapy machines are not available on **PAY PER USE** basis which makes it unviable when it comes to unit economics.

(To know more about Apollo's proton therapy & equipment refer attached link : https://www.iba-protontherapy.com/)

Exhibit 19: HCG's market share on key metrics

Particulars	HCG	Industry	HCG Market Share (%)
Total Oncology FY23 Revenue (Rs.Mn)	16,808	2,37,000	7.1
Total New-patients (No.)	74,000	20,00,000	3.7
Total Oncologists (No.)	400	4,835	8.3
New Patient to Oncologist Ratio	185 patients per oncologist	276 patients per oncologist	
Total Patient to Oncologist Ratio	400 patients per oncologist	1000 patients per oncologist	2.5x oncoligst density vs industry

Key Information	No.
Total Oncologists	400
Total Operational Beds	1,702
Total LINAC's	31
Total Operation Theaters	66
Total PET CT Scanners	14

Source: Company Annual Report, Ernst & Young, Call for Action: Making quality cancer care more accessible and affordable in India, Dalal & Broacha Research

Did you know?
Saddening fact on mortality due to cancer
In last 3 years more people died due to cancer than COVID across the world. In 2020, there were 8 to 9 lakh deaths due to cancer in India, an estimated economic burden in terms of GDP losses is in the range of US $11 billion, that is 0.4% of national GDP in 2020.

New cases a right matrix to track cancer patients

Close to 3.7 Lac+ new patients have registered in oncology in last 5 years.While HCG has been able to maintain its lion's share at ~70k new patients.***Refer Above Exhibit No.19 for more details.***However,when it comes to oncology more than new patients one should look at new cases because a single patient could require 3 modalities (diagnostic,radiation or surgical/medical).Each modality is different revenue generator.So, incase of HCG new cases (patient under-going different modalities) are usually 1.5-2x of new patients taking the count to ~1.5 lac cases p.a.

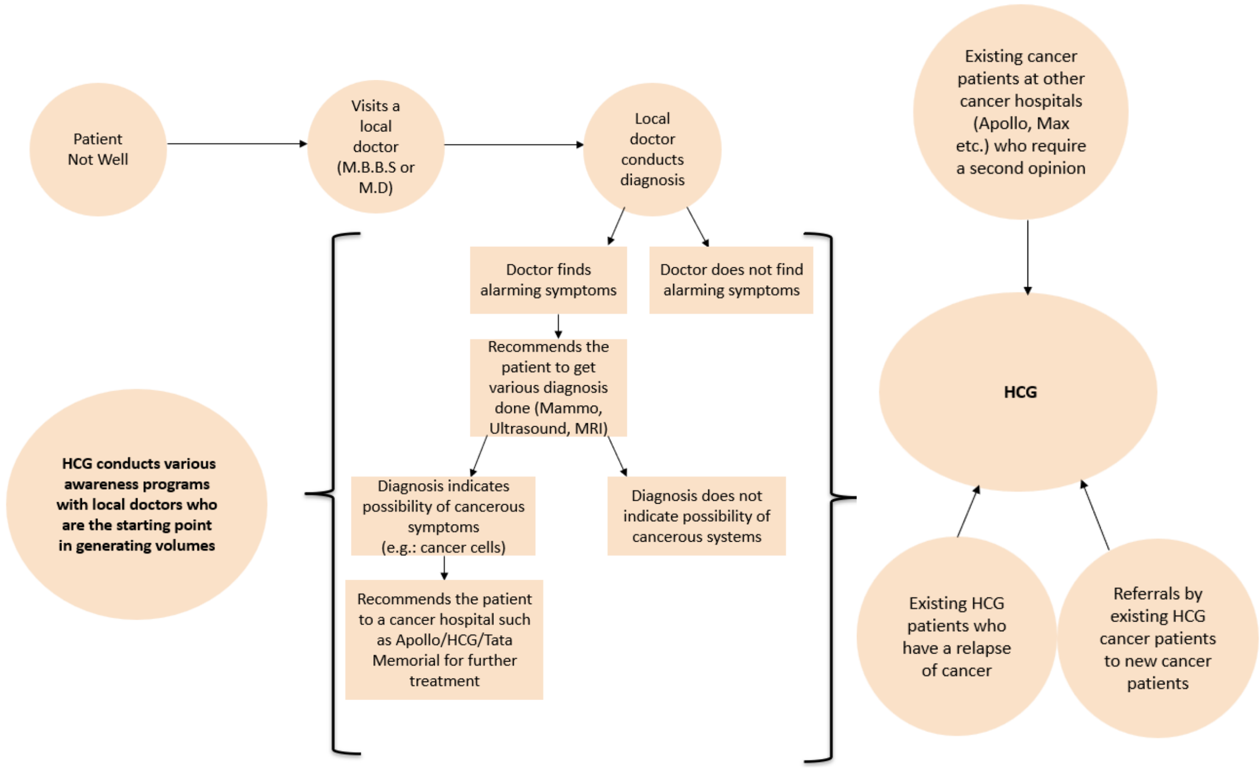
As per national cancer registry there are ~13 Lacs+ cancer patients in India but in reality this number could be upward of ~27 Lacs+ patients.Lot of patients are not registered under the national cancer registry.

HCG is not just a choice of hospital for patients but a choice of consultants as well

(What motivates oncologists to join HCG Hospital vs Apollo Hospital or any other hospital)

HCG being a research oriented organization,learning is a key driver.Oncologists prefer HCG over other hospitals only because of cross learning,research capabilities & not just because of monetary benefits.HCG's average pay to consultants would be 5-10% lower than the likes of Apollo or Max hospital but in terms of stickiness & loyalty HCG is much ahead because the motivating factors are different unlike other doctors.Usually in a multispeciality hospital there is one department of oncology where the doctor works independently & knowledge sharing is limited vs a cancer focused hospital where the core focus is oncology.So HCG is not just a choice of hospital for patients but a choice of consultants as well.HCG has by far the largest oncologist network in India with 400+ oncologists.

Exhibit 20: Epicentre of Volume Generation & how is HCG trying to transform it



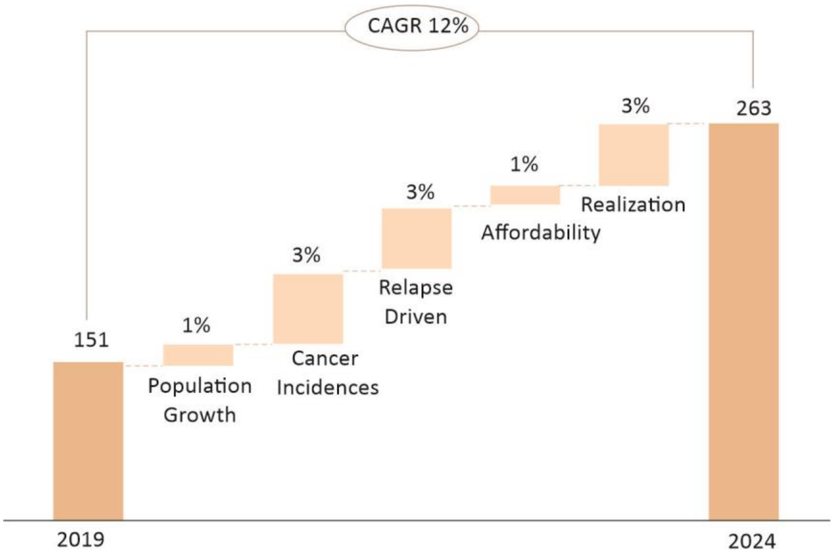
Source: Company, Dalal & Broacha Research

There are various channels through which patient lands to HCG Hospital.Typically,1st step in the process is that patient may have any illness or reaction that makes him visit to his local doctor (M.B.B.S or M.D.) where the doctor may suggest to get various diagnosis done & on the basis of report if the doctor finds something alarming he may further send patient for advanced diagnosis (eg: biopsy) & based on the report,local doctor will recommend advanced treatment if cancerous cells are detected.They may recommend to visit any of the hospitals such as Apollo,HCG or any other hospital.

HCG does various awareness programs with various local doctors who are the start point in generating volumes.Increased awareness drives more volumes as screening of cancer becomes faster & visibility of HCG brand increases.Over, the past few years HCG has focused on creating strong sales funnel.Also HCG is leveraging digital technology to enhance its sales funnel.USP of HCG is that it does not incentive monetarily to any doctor who sends patients to HCG hospital,while the recommendation of doctors is service/outcome driven & not incentive driven which builds long term stickiness with doctors.

Also, there are various other ways by which patient lands to HCG,typically when patient has taken opinion in one hospital & want to visit another Dr/hospital for 2nd opinion (via secondary research),also existing HCG patient become a source of reference & the last source of volume are existing patients who are facing relapse.

Exhibit 21: Cancer Industry to grow @ 12% CAGR & outpace other specialities ; Various gaps that will drive cancer care in India



Source: Company, Dalal & Broacha Research,Data in Bn Rs.

Cardiac & oncology are 2 specialities which garners highest growth rate amongst various specialities.Oncology is seeing a paradigm shift as day by day lifestyle related cancer cases are increasing drastically & thus growing at faster pace than other specialities.If we were to rank fastest growing specialities in India then cardiac would be no.1,oncology at no.2 & neurology at no.3.

There are 4 major gaps which will increase cancer incidences going forward :

Gap 1: Increasing awareness & prevention

Awareness levels about tobacco-related cancers are high in India, however awareness about other risk factors such as HPV, alcohol and obesity are still not very prevelant and implementation of preventive measures will drive cancer incidences going forward.

Gap 2 : Increased screening & diagnosis

Only 1% or less than 1% of India's population is covered under screening programs for major cancer types, such as oral cancer, breast and cervical cancer, and more than 2/3rd of cancer patients get diagnosed late at advanced stages of the disease.Affordable screening & diagnosis is need of the hour. Out of the 480 comprehensive cancer care centers available in the country, about 40% are concentrated in metros and state capitals. Country needs to add another 570 comprehensive cancer care centers by 2030 to meet the demand.

Gap 3 : Access to treatment

Population living in more than 70% districts in India do not have access to comprehensive cancer centers.

Gap 4 : Reducing outcome Gap

Challenge of growing incidences is further intensified due to suboptimal outcomes compared to global counterparts across all major cancer types, due to all other gaps.

HCG has been a pioneer and the leader in addressing all these gaps with their 21 comprehensive cancer centers, largest in India. With 2/3rd of these centers in non-metro locations,they are trying to make quality cancer care more accessible and affordable in India since their inception.

Insurance penetration to drive oncology growth

As the penetration of insurance increases in India so will the cancer care market grow.Also the coverage of various modalities,diagnostic & medicines have increased over the years making it affordable to patients.Schemes like Pradhan Mantri Jan Arogya Yojana,rising per capita income are other major drivers of cancer care in India.

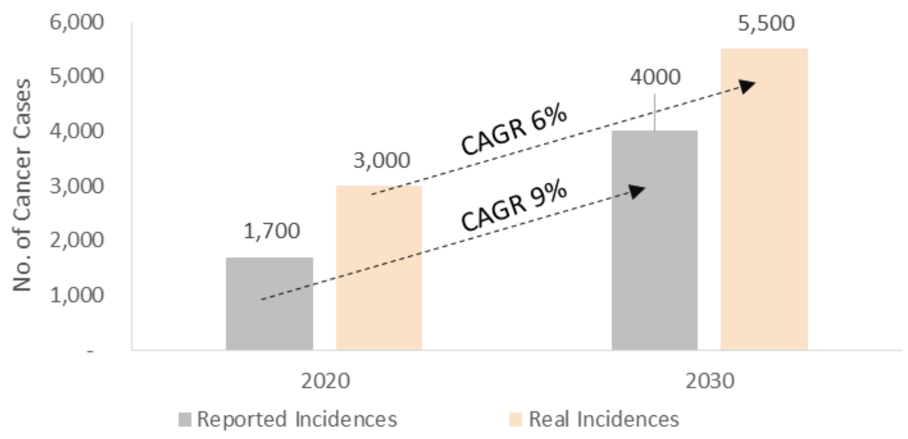
Are all cancer therapies & medicines covered under insurance policies?

Yes,barring 2-3% of overall cancer expenses/services,almost everything is covered under insurance policies *(Refer FAQ for more details)*

Recent Update on cancer by Pfizer (14-03-2023)

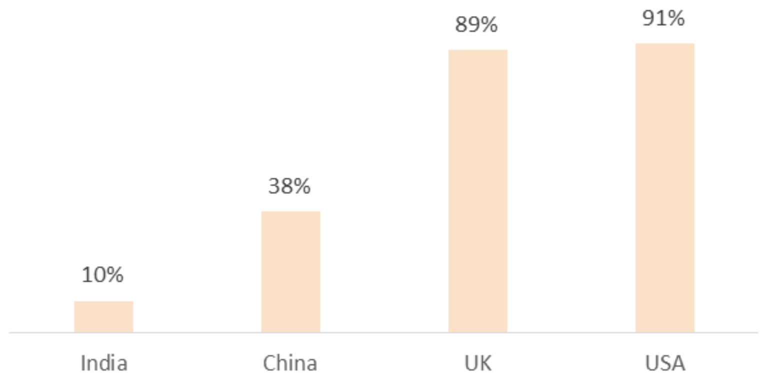
Pfizer announced cash acquisition of Seagen, a cancer specialist biotech for $43 Bn - largest pharma M&A in 3

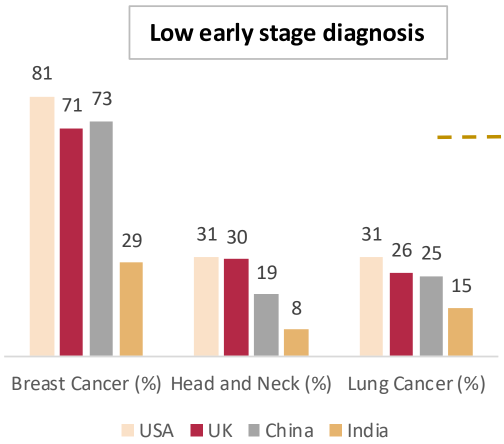
Exhibit 22: Increasing incidence of new cancer cases in India

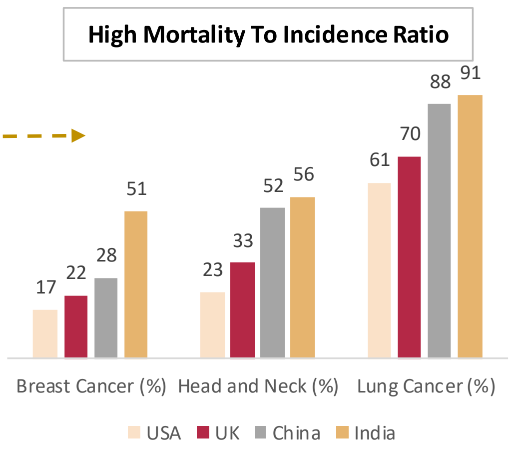


Source: Company, Dalal & Broacha Research

Exhibit 23: Population coverage by national cancer registeries



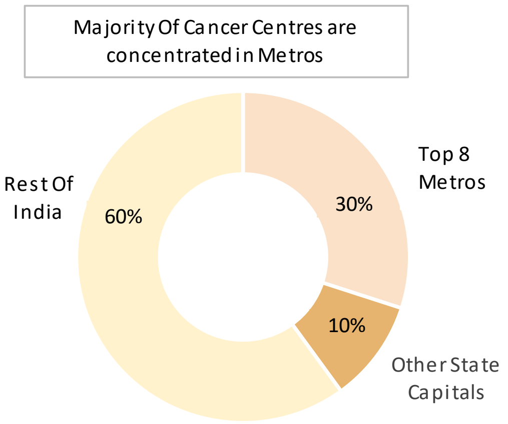


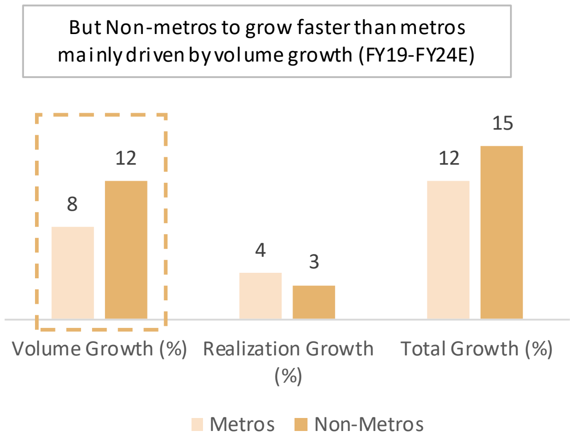


Source: Company, Dalal & Broacha Research

As per national cancer registry there are ~13 Lacs+ cancer patients in India but in reality this number could be upward of ~27 Lacs+ patients.Lot of patients are not registered under the national cancer registry.

Exhibit 24: Majority Of Cancer Centers are concentrated in Metros

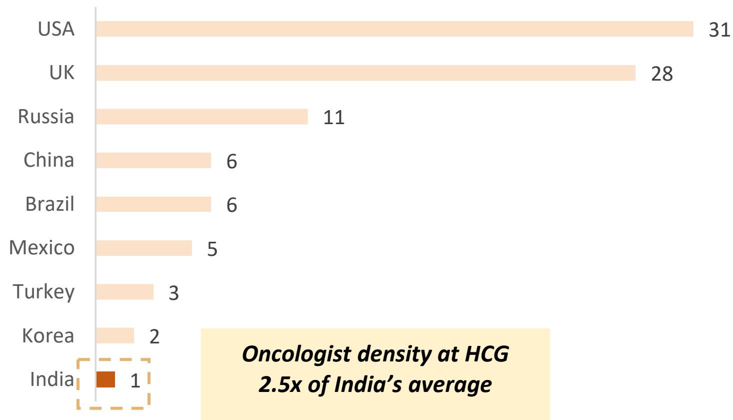


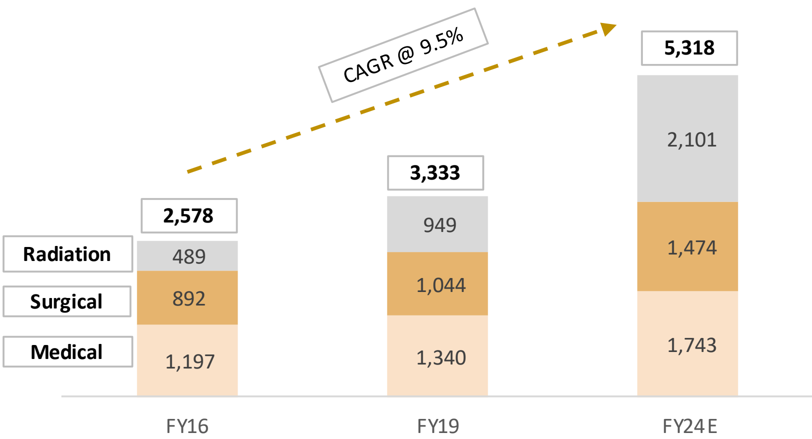


Source: Company, Dalal & Broacha Research

HCG is trying to increase its penetration in non-metro region where cancer centres are not present but growth is higher than metro cities.

Exhibit 25: Oncologists Per '000 Cases & Number Of Oncologists In India





Source: Company, Dalal & Broacha Research,Data as on FY22

Oncologists are the drivers in cancer care.HCG has the high density of oncologists at 2.5x of India's average.

Largest B2B share in referral network of doctors without sharing any commission/incentives to doctors referring patients

HCG is a leader across 13 out of 18 cities in terms of revenue because it has been able to build largest share in B2B referral network of doctors without sharing any commission/incentives to doctors referring patients.Unlike other hospitals HCG has created a strong moat in doctor referral which is the starting point in volume generation.Doctor referral is based on clinical outcome & not incentives which helps build long term stickiness at entry level which is hard to replicate for other hospitals.(Eg : If hospital A reduces incentive of Doctor X,he may refer patient to hospital B which is unlikely incase of HCG). ***Refer Exhibit No.19 for further details which explains what motivates doctor to refer patients to HCG despite HCG not sharing any monetary incentive/commission***

Over the years B2B referral had remained the largest source of volume generation forming close to 90% of overall volume.Lately,HCG has started spending on sales & marketing to increase its brand awareness.

Primary clinicians providing primary or secondary care have a patient base.If the doctor's clinic falls in the catchment area of HCG they would recommend HCG on basis of clinical outcome because he would want his patients to get the best of treatment & service as he may be a family doctor for lot of patients that he is referring to & he would not want to risk his name by referring to wrong hospital or doctor just to increase his incentives because his reputation would be at stake.

Also when doctor sends any patient to multi-speciality he may have fear of losing the patient as in multi-speciality hospitals there are various other specialities which could be conflicting with his own practice vs if he sends the patient to HCG where there are super-specialist which are expert only in oncology there is no fear of losing the patient at all.

Eg : Mr.A who is a patient with Dr.Local (Urologist) was referred to oncologist at Apollo,now at Apollo hospital there will also be a urologist which the oncologist may suggest internally & there are chances that Dr.Local may lose that patient forever.

Lowest attrition rate of Oncologists

HCG has one of the lowest attrition rate in industry & key clinicians have been there with the company for ages.HCG does not chase staff clinicians for practice which is a differentiator.Doctors have come to HCG,they have practiced for years with them & by-default they have been star doctors along the way.Having seen how HCG practices oncology throughout their journey,it would be difficult for them to directly get out of the ecosystem thereby bringing more stickiness.There are experienced oncologists with the company for 15-20 years+ at centre of excellence.

HCG has built its empire on partnership & private equity contribution.HCG has total 7-8 doctor partners who have some shareholding in those facility.Ahemdabad facility has 24% shareholding owned by surgical oncologists there.

Why HCG doesn't give ESOPs to its doctor?
In India typically all doctors are on consultancy basis vs being classified as employee,as this structure is tax beneficial to doctors & ESOPs are given to employees & not to consultants

Exhibit 26: Peer Comparison of Oncology revenue by different hospitals along with revenue per centre

Companies	Presence across	Cancer Centres	Revenue from Oncology (Rs.Mn)	Revenue per centre (Rs.Mn)
HCG	Metros & Non metros	21	13,357	636
Max Healthcare	Single Region	7	8,000	1,143
Apollo	Multiple Metros	13	6,200	477
Narayana Health	Multiple Metros	7	5,230	747
Fortis	Multiple Metros	8	4,172	521
TATA Memorial	Single Region	1	3,900	3,900
Omega Hospitals	Multiple Metros	8	2,400	300
KIMS	Single Region	1	800	800

Source: Company Investor Presentation, Dalal & Broacha Research

Note : Only oncology revenue considered for all hospitals.Approximate revenue for peers ; FY21 Revenue considered for Tata Memorial Hospital-Mumbai, Apollo and Omega; FY22 revenue is considered for all others ; Oncology share of total revenue is assumed to be 100% for Tata Memorial and Omega hospitals

Exhibit 27: CVC Deals & Purchase Price History

Date	Shareholder	No.of Shares	Price (Rs.)	Cost of Purchase (Rs.Mn)	% of Overall Shareholding
29-07-2020	CVC Capital Partners (Aceso Company PTE Ltd)	3,65,73,455	120	4,389	29%
10-09-2020	CVC Capital Partners (Aceso Company PTE Ltd)	2,60,48,478	130	3,386	50%
29-03-2021	CVC Capital Partners (Aceso Company PTE Ltd)	46,83,177	185	866	54%
07-12-2021	CVC Capital Partners (Aceso Company PTE Ltd)	1,15,03,468	130	1,495	57%
01-07-2022	CVC Capital Partners (Aceso Company PTE Ltd)	17,50,500	286	500	58%
	Total	**8,05,59,078**	**132**	**10,637**	

Average Cost of Purchase of CVC (Rs per share)	**132**
XIRR for CVC (%) - Based on 10th Mar Closing Price	**~40%**

Note : Numbers are based on our internal calculations

Source: Company, Dalal & Broacha Research,BSE Website

CVC bought a controlling stake in 2020 when company was going through financial crises.HCG went into robust expansion phase (heavy capex plans of ~Rs.10,000 Mn) during 2016 to 2020 & then covid hit which led to low volumes & leveraged balance sheet.Then CVC came in & above are details of their stake purchase & their average cost of acquisition.CVC being a private equity player usually have a time frame of 5-7 years. Currently they do not have plans to exit the company.Their investment period at this point stands around 2-2.5 years.They have not signed any fixed exit contract with HCG.

Founding promoter Dr.Ajaikumar plans to keep his stake at same level (~13% stake) right now & would not like to increase as he is at 71 years.

Who is CVC Capital Partners?

CVC Capital Partners is a Luxembourg-based private equity and investment advisory firm with approximately US$133 billion of assets under management.

Exhibit 28: Treatment cost comparison betweeen USA & India

Treatment	USA (PPP Adjusted) Rs. Per Patient	India (Rs. Per Patient)
Chemotherapy	6,15,000	1,95,000
Surgery	6,60,000	80,000
Radiation	4,80,000	80,000

**USD/Rs. exchange rate is assumed at Rs. 80/USD*

Source: Company, Dalal & Broacha Research

Exhibit 29: Single speciality (HCG) vs Other Multi-speciality Hospitals (Key Differentiating factors)

Differentiator 1 : Clinical outcomes a major driver in selecting doctor/hospital ; Survival rate a lead indicator of clinical outcome

Clinical outcomes are different in different hospitals based on the in-depth practice (specialization,sub-specialisation),research,machines,technology,tools,knowledge sharing,domain expertise & various such factors.However,incase of oncology there is no particular database of outcomes whether HCG hospital is better or any other hospital has better outcome.HCG does extensive research & caters to large database of patients which allows it to stay one step ahead compared to other hospitals.Based on the sheer volume that HCG caters our belief is that the clinical outcomes at HCG should be far superior compared to other hospital when it comes to oncology.HCG is trying to collate such data points to attract more volume to his centers.Ultimately,success ratio/survival rate matters incase of oncology which is one of the major driving force when it comes to selecting doctor & hospital.

Survival rate a lead indicator of clinical outcome.Eg : A person who has stage 4 cancer, a true measure of clinical outcome would be how long can he live his life in least pain.Survival rate are usually defined on 5/10 year period basis.

The five-year survival rate for breast cancers at HCG stands at 86.9%, which is the highest in India. A case study published in Harvard Business Review has reported that HCG's five-year survival rates for breast cancer cases are on par with the global standards which makes it a preferred choice for patients that too at 1/10th the cost in USA.

Refer more on https://www.hcgoncology.com/types-of-cancers/breast-cancer/

Differentiator 2 : Sub-specialization is need of the hour

HCG has built sub-specialization expertise providing very high value proposition.

With ever increasing complexity of cancer & need for accurate treatment there are various sub-specializations which have emerged.There are close to 53 different types of cancer each requiring unique treatment & know-how.

As awareness & screening increases there will be various branches which will be required to be catered.Multi-speciality hospitals have one department of oncology vs a full fledged oncology centric approach.For eg : At HCG's Centre of Excellence there are close to 70-75 oncologists with organ specific super-specialised oncoligsts having deep domain expertise vs multi-speciality hospitals having limited or no super-specialist.

On contrary to other specialities which are standardized & outcomes are known, oncology requires special care as it needs to be treated accurately at the outset otherwise there are chances of relapse.

Eg : Person suffering from cardiac arrest may get an angiography done & if the reports are not normal there are 2 options given to the patient of either bypass or angioplasty vs person suffering from breast cancer could have various therapies (lumpectomy,mastectomy,implant reconstruction etc) which could be recommended by different specialists having different outcomes

Differentiator 3 : Largest Tumor board in India

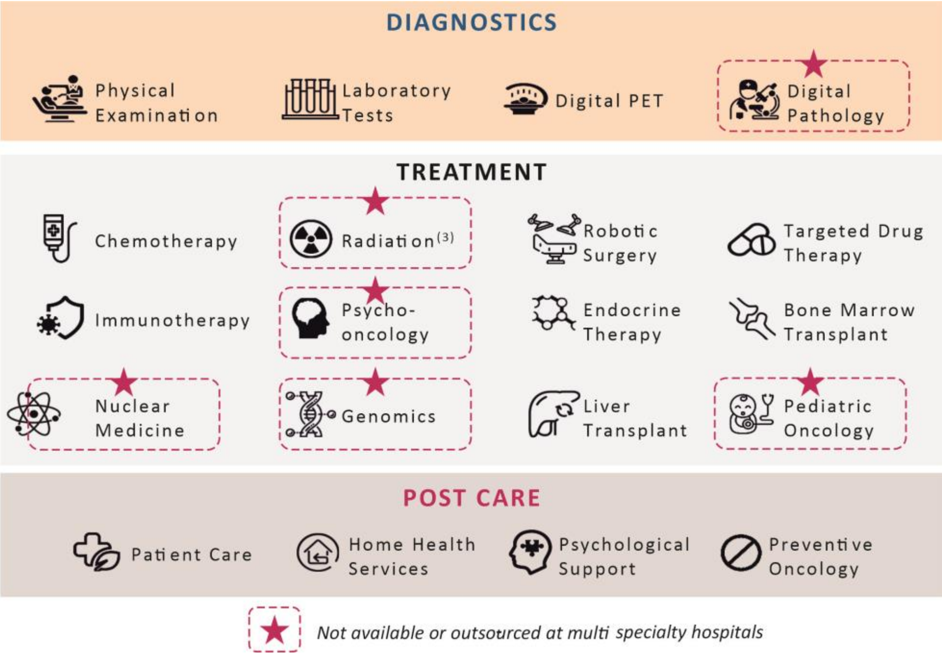
Tumor board is a unique approach whereby group of oncologists meet every week to discuss critical case & decide what treatment needs to be given to a particular patient taking into account various factors.This helps in enhancing the accuracy level & outcome levels are far better.HCG has the largest tumor board in India which is a key differentiating factor in itself.

A unique case in Cuttack may have past experienced in Banglore facility (Eg:Various outcomes of BBB negative breast cancer & what protocols one needs to follow if the same was experienced by a Dr in Bangalore).Tumor board helps facilitate knowledge sharing & increases outcome level which is very important with oncology speciality.

Differentiator 4 : Pioneers of research in India

Very few institutes like HCG & Tata memorial in India are focused on R&D & academics.HCG has been at the forefront of Research & development when it comes to cancer research.HCG till date has published close to 756 research papers.Also on the academics front HCG runs various fellowship programs thereby creating a solid pipeline for clinical talent.

Differentiator 5 : All modalities under one roof

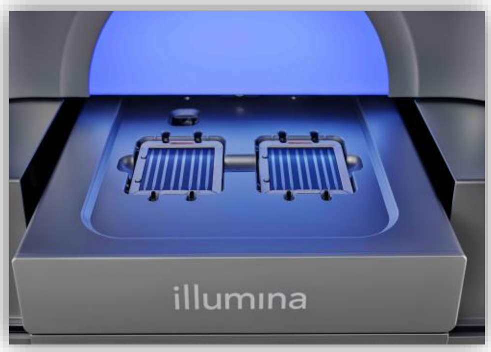


Source: Company, Dalal & Broacha Research

Most of the multi-speciality hospitals have one department for oncology but they lack comprehensive cancer care centers.HCG has dedicated 21 comprehensive cancer care centers in India which provides all modalities (diagnostics,radiotherapy & medical/surgical oncology) under one roof.

HCG has been at the forefront in bringing newer & better cancer technology to India ***(Refer Exhibit No.17 & 18 for more details).***

Differentiator 6 : Largest gene sequencing in the country (Genomics Lab)



Source: Company, Dalal & Broacha Research

HCG has taken a leadership role in genomics-driven tumor boards and gene-profiling.HCG has completed 500 gene sequencing for over 1,500 patients, which is the largest in the country till date.This has given insights into patient-centric approach, particularly for advanced and recurring tumors, not only from India, but from Africa and Middle East, making HCG a destination for cancer care.This approach helps in better outcome & indirect more patient referrals.

(Refer Link to know more about genomics : https://www.hcgoncology.com/technology/genomics-and-molecular-diagnostics/ ; Also Refer FAQ for more detail on genomics)

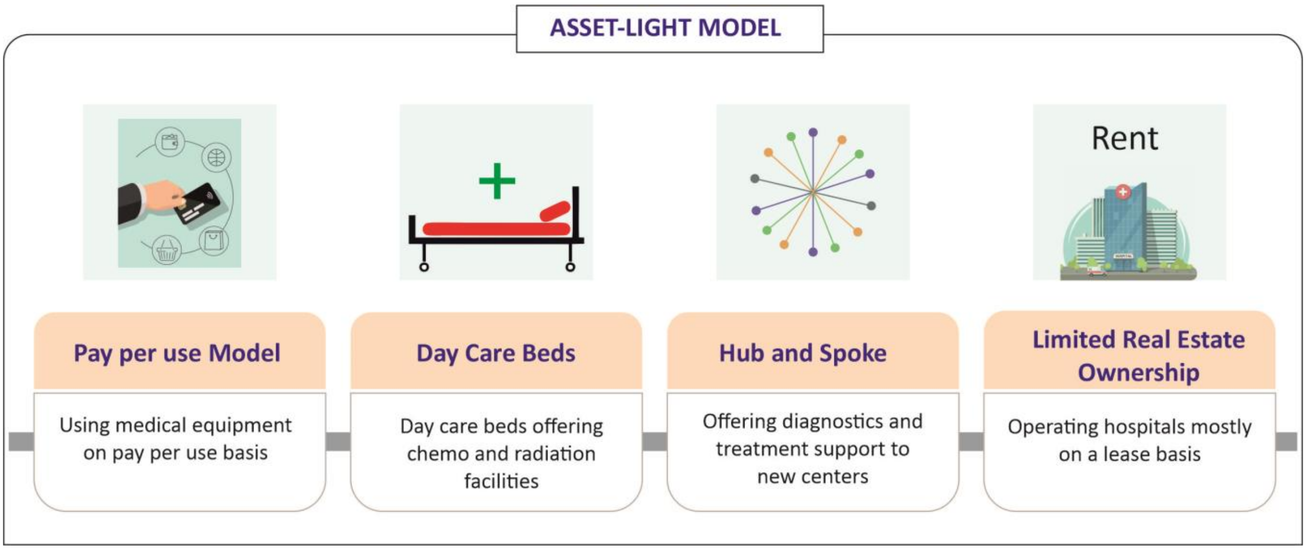
Ever wondered : Is it possible for other big hospital chains to set-up genomics laboratory?

For HCG or any other cancer care centre equipment can never be differentiator but the **ecosystem around it is the key differentiator**.Average cost of setting up genomics laboratory is meagre Rs.100-150 Mn which any other hospital in India can replicate.Also the quantum of data that HCG has with respect to cancer patients is nowhere closer to any other hospital.Incase of sequencing more the sample higher are the chances of accuracy.

However the key differentiator is the outcome analysis that comes of genomic test.Experts like pathalogists,molecular diagnostics,clinician discusses the report & there is lot of customization which is done at HCG & then treatment protocols are decided whereas in multi-speciality it is usually a decision of one doctor or few doctors,also they are less domain experts or sub-specialization doctors practising in each domain.

Usually in absence of genomics doctor gives treatment based on organ specific issue without knowing the root cause of it & cancer is gene based disorder,Based on genomics result molecular diagnostic person may recommend xyz therapy whereas clinician would counter-argue saying this therapy has not worked in the past incase of similar patient.This is key differentiator when it comes to HCG vs other hospitals.

Exhibit 30: Combination of various asset light strategies to drive RoCE



Source: Company, Dalal & Broacha Research

RoCE to be driven by various measures such as :

1.Pay per use for machines (Renting machines rather than owning it)

2.Focus on higher utilization of day care beds which do not require incremental capex

3.Hub & spoke model to leverage common brand & create operating leverage

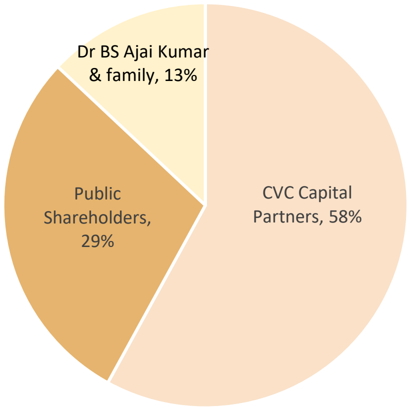
4.Leasing centers rather than purchasing to deploy more capital on opex rather than capex

HCG follows MD Anderson & Sloan kettering model for quality treatment with best in class diagnostic & medical protocols

Did you know?

MD Anderson has been ranked as the top cancer hospital in the world for many years, and it is at the forefront of developing and using cutting-edge diagnostic technology for the best possible outcome. **Memorial Sloan Kettering Cancer Center** is the largest, oldest and best cancer center in the world.Dr Ajai obtained a MD at MD Anderson Hospital in Houston, Texas.

Exhibit 31: Shareholding Pattern



Shareholder Type	Shareholder	Percentage Of Shareholding
Promoter & Promoter Group	Acesco Company Pvt Ltd (CVC Capital Partners)	58
	B S Ajaikumar & Family	13
FIIs	FIIs like IFC,Wellington Trust Company	7
DIIs	Various Funds like TATA India Pharma & Healthcare Fund	4
Public/others		17
	Total	100

Source: BSE Website(Q3FY23), Dalal & Broacha Research

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FY22 Annual Report Analysis

New Products/Improvements/Expansions

New Acquisitions:

- **Suchirayu in Hubli (Nov 2021)**: Increased stake from 17.7% to 78.6% for an additional consideration of Rs.330 Mn (for 60.9% stake)

  Beds: 110 beds (Potential to scale upto 250 beds)

  EBITDA (March 2022) : ~20%

  Type Of Deal : Cash

1. **Shiv-Sun Medical Services LLP (October 2021):** Acquired 26% stake of minority partners for HCG Sun Hospitals LLP in Rajkot making it a wholly owned subsidiary of HCG for Rs 25 million.

   Type Of Deal: Cash

Stake Sale of Strand Life Sciences Pvt Ltd :

2. Sold 34.5% (entire) stake in Strand Life Sciences in Sept 2021- Cash Inflow: Rs 1,577 mn
3. Bought back labs and clinical research business through business transfer agreement which will help provide better quality cancer care and offer scaling up and margin improvements Cash Outflow: Rs 810
4. **Net Cash inflow: Rs 830 mn** (including set off of Rs 70 mn towards receivables).

Expansion:

Greenfield

- Ongoing capex for two greenfield expansions - INR 186 mn (balance in FY23) and INR 836 mn (FY24) in Bengaluru (20 beds) and Ahmedabad (150 beds) respectively. This is to be operational in Q4FY24 and Q1FY25 respectively.

Brownfield

- Actively looking out for brownfield expansion opportunities in UP and Punjab.

Business Model Improvements:

- HCG has shifted their investment strategy from equipment purchase to pay per-use and most of their hospital facilities are leased.

Technological Improvements:

- Acquired Next Generation Sequencer. Soon acquiring circulating tumor cells (CTC platform). Used in early detection of recurrent tumors and genomic guided therapy.
- Enterprise RIS PACS in collaboration with Siemens for AI based precision imaging which will enhance research capabilities.
- AI enabled adoptive radiotherapy in Centre Of Excellence (COE), a first in India.
- Partnered with Microsoft on a mixed-reality platform to help the doctors in Tier II and Tier III perform complex surgeries effectively under the guidance of specialists from hub centers

Other Improvements:

- LINAC's and Tomography machines to tier 2&3 cities and towns.

Product Profile

HealthCare Global Enterprises Limited (HCG) provides comprehensive cancer diagnosis and treatment services including radiation therapy, medical oncology, and surgery. It also provides tertiary care, infertility treatment and advanced screening and diagnostics.

Sales breakup, by type of speciality (FY22):

- Oncology: 96%
- Fertility: 4%

Sales breakup, by type of patient (FY22):

- Domestic Patients: 98%
- International Patients: 2%

Key Products

- Diagnostic Services
- Radiotherapy Services
- Medical/Surgical Oncology Services

Business Model

HCG operates through a hub and spoke model, comprehensive cancer care centers and partnerships.

Comprehensive Cancer Care Centers: 22 comprehensive cancer care centers providing complete diagnosis and treatment (radiation, surgical and medical oncology) services.

Hub and Spoke Model: HCG offers services tier 2/3 cities through its hub and spoke model. COE in Bangalore serves as a hub which consists of high-end equipment and the most highly trained oncologists. At the spoke facilities, less specialized doctors provide less specialized services. The hub-and-spoke approach is facilitated using technology—such as telemedicine, which enables the remote delivery of health care.

Partnerships: HCG also operates through partnerships with local specialist physicians and multi-specialty hospitals, which allow them to leverage the position and reputation of their partners in the local communities.

Industry Outlook

- India's cancer care industry is estimated to grow from **Rs 151 billion in 2019 to Rs 263 billion by 2024 at a growth rate of 12%**, driven by population growth, new cancer incidences, relapses, affordability and better realisations.
- India ranks 3rd in terms of reported cancer cases every year after USA and China. The reported cases in 2022 were approximately 20 lakhs. It is estimated that the actual cases are 1.5 to 3 times higher than the reported cases.
- There is lack of awareness and screening of cancer cases in India. The screening rate in India is a mere 5% of the population, a negligible proportion when compared to global peers. Only 29%, 15% and 33% of breast, lung and cervical cancers respectively were detected in stage 1 and 2 causing complications in procedures and financial burden to victims.
- India faces a shortage of equipment used in the screening, diagnosis and treatment of cancer in India. India also faces a shortage of oncology professionals.

Particulars	Actual (Supply)	Required (Demand)
Cancer Care Centers	484	656
Radiotherapy Equipment	640	1,305
PET-CT	360	480
Medical Oncologists	1,969	5,021
Surgical Oncologists	1,905	2,690
Radiation Oncologists	3,360	3,242

- The government is improving its role in the Indian healthcare industry by launching various schemes that aim to offer equitable access to healthcare. One of the most promising schemes launched by the government is Ayushman Bharat Pradhan Mantri Jan Arogya Yojana (AB-PMJAY), one of the largest healthcare schemes in the world, with a cover of upto INR 5 lakhs per person.
- India has significantly improved cancer treatment processes over the years. Some of the improvements in the cancer treatment process include the use of genomic guided immunotherapy, liquid biopsy, Artificial Intelligence (AI), Adaptive Radiation Therapy etc.
- India offers cancer treatment at a cost which is significantly cheaper than its global counterparts and yet has a high success rate making it a serious contender to become the cancer care hub of the world

Employees

- Permanent - 6,208 (5679 On roll Employees & 529 Fixed Consultants)
- Nurses- 1,922
- Resident Doctors- 170
- Paramedical Staff - 1,287
- Oncologist, Radiologist, Pathologist & Other Specialist- 432
- Fellowship- 32

Geographical Breakup

- Karnataka: 36%
- Gujarat: 26%
- Maharashtra: 17%
- East India: 9%
- Andhra Pradesh: 8%
- Others: 4%

Capex Plan:

- The company is estimated to have low future capex requirements owing to an asset light business model. The following is the planned CAPEX:

Particulars	Capex incurred till 31st Dec 2022 (Rs.Mn)	Expected Capex between Jan 23 to Mar 23	Expected Capex for FY24 (Rs.Mn)	Total Planned Capex (Rs.Mn)	Expected date of Operations
Ahmedabad – Phase II	84	132	636	852	Q1FY25
Whitefield (Extension of Bangalore - COE)	10	40	200	250	Q4FY24

Other Key Data & Metrics:

Cancer Care Metrics, As On 31st March, 2022

- Comprehensive Cancer Centers- 22
- States Present- 9
- Nurses- 1922
- Oncologists- 300+ (Added over 40 Oncologists in FY 2022)

Reproductive Medicine Metrics, As On 31st March, 2022

- New Couple Registration- 4,633
- IVF Cycles in FY 22- 1,747
- IVF Specialists- 27
- Embryologists- 9

Clinical Laboratory Metrics, As On 31st March, 2022

- Clinical Trials- 80
- Genomic Tests Performed- 1,500+
- Specialized Test- 26,500
- Published Citations- 7

Multispecialty Hospitals Metrics, As On 31st March, 2022

- Doctors- 99
- Nurses- 491

- Hospitals-4

Financial And Operational Metrics, As On 31st March, 2022

- Revenue- Rs 13,978 Million (increased by 38% y-o-y)
- Operating EBITDA- Rs 23,797 lakhs (increased by 88% y-o-y)
- Number Of Beds- 1,702
- Average Length Of Stays- 2.28 days
- Operating EBITDA Margin- 17% (Was 12.5% in FY 2021)
- Average Occupancy Rate- 58.3% (increased from 48.4%)
- New Patient Registration Across Cancer Centers- 73,420 (increased from 60,734)
- Average Revenue Per Occupied Bed- Rs 36,697 (Up from Rs 32,632)
- 73,420+ new patent registrations including 387+ new international patent registrations

Where is Healthcare Global Enterprises placed in the cancer care market?

- Healthcare Global Enterprise (HCG) is the leader in the Indian cancer care industry with 22 comprehensive cancer care centers (including 1 center in Kenya), generating more revenues from oncology than any other cancer care provider in India.
- Healthcare Global Enterprise (HCG) has the largest oncologist network in India with over 300 oncologists.
- Healthcare Global Enterprise (HCG) has the highest number of cancer equipment used in India. It has 31 LINACs, ahead of 17 LINACs owned by Apollo hospitals,market leader in healthcare.
- Healthcare Global Enterprise (HCG) is switching to an asset-light business model. It is opting for pay-per use machines in order to reduce capital expenditure, leading to a higher return on capital employed (RoCE). It is able to opt for pay-per use only because of the high patient volume it generates.
- Healthcare global enterprises (HCG) has performed over 1,500 genomic tests. Superior understanding of genomics is expected to improve clinical outcomes, giving HCG an edge over its competitors.
- HCG’s multidisciplinary tumor board approach is unique from that of its competitors.

Director Remuneration

- Mr Meghraj Arvindrao Gore (Director & CEO) – Rs 50.25 Mn
- Dr BS Ajaikumar (Executive Chairman)- Rs 25.90 Mn

Auditors

- Statutory Auditors: M/s. B S R & Co. LLP
- Audit Fees: Rs. 13.55 Mn

Outlook & Valuation

HCG is a story of how a doctorpreneur created a world-class cancer hospital chain.HCG is a brainchild of an oncologist whose radical & multi-disciplinary approach towards cancer treatment combined with highest levels of research & technology in India is killing cancer affordably.Unlike other specialities oncology has to be treated right way the first time,where tumor is very notorious because there are chances of relapse.HCG is the largest oncology focused hospital in India with PAN India presence.

Company is truly at inflection point.Company has delivered positive Adjusted PAT (Rs.+321 Mn) in 9MFY23 from Adjusted loss of (Rs.-1,144 Mn) in FY21.HCG has continued its streak of highest ever revenue of Rs.4,247 Mn for the 8th quarter (Q3FY23) in a row & highest ever EBITDA of Rs.755 Mn (Q3FY23) for 7 quarters in a row.However,on cashflow front company has always been positive with CFO expected to touch ~Rs.2,500 Mn by FY23E.Also, company is revamping its business model from owing the machines to renting it (Asset heavy business to asset light leading to higher RoCE).Various other initiatives such as,ramping up of emerging centres, hiring big4 to drive business excellence, low incremental capex,change in go-to-market strategy,integration of various departments & centres,digital transformation & brand building activities are key drivers of revenue & growth. Also HCG is actively looking for smaller acquisitions to drive future growth.

EBITDA is expected to expand 200-300 bps by scaling emerging centers which are at ~9% EBITDA margin vs mature centers at ~25% EBITDA margin & company level EBITDA margin of ~21% (before allocation of corporate expenses). With multiple levers in place HCG is all set for turnaround & growth.

HCG is currently trading at 10x FY25E EV/EBITDA multiple.We value HCG at 15x FY25E EV/EBITDA multiple to arrive at a target price of Rs.391 (HCG has traded at an average forward EV/EBITDA of 17.6x over the past five years).

Particulars	FY25E
EBITDA - Post IND AS	3,871
EV/EBITDA (x)	15
EV	**58,067**
Less: Net debt,Leases & minority	3,750
Implied Mkt Cap	**54,318**
Value per share	**391**

Various reason for 15x EV/EBITDA Multiple :

Industry average EV/EBITDA is 22x on TTM Basis whereas HCG being single speciality trades at much deep discount compared to peers

HCG has traded at an average forward EV/EBITDA of 17.6x over the past five years

Hospitals in developed market trade at 9x EV/EBITDA on TTM Basis whereas in emerging markets because of the growth rate hospitals are expected to trade at much higher multiple

Peer Comparison

Company Name	CMP (₹)	Mcap (₹ Mn)	Revenue (₹ Mn)			EBITDA (₹ Mn)			PAT (₹ Mn)			Adj EPS			PE (x)			EV/EBITDA (x)			Revenue CAGR (FY23-25E)	EBITDA CAGR (FY23-25E)	EPS CAGR (FY23-25E)
			FY23E	FY24E	FY25E	FY23E	FY24E	FY25E	FY23E	FY24E	FY25E	FY23E	FY24E	FY25E	FY23E	FY24E	FY25E	FY23E	FY24E	FY25E			
Apollo Hospitals	4,304	6,19,014	1,68,000	1,96,176	2,27,151	21,344	27,136	33,251	8,478	12,357	16,650	59.1	86.1	115.9	72.8	50.0	37.1	30.4	23.9	19.5	16%	25%	40%
Max Healthcare	470	4,56,207	58,835	64,973	76,657	15,892	17,830	20,824	11,270	11,828	13,830	12.0	12.6	14.6	39.2	37.4	32.1	28.6	25.5	21.8	14%	14%	10%
Fortis Healthcare	262	1,97,836	63,492	71,400	79,355	11,374	13,376	15,366	5,417	6,700	8,179	7.1	8.9	10.8	36.9	29.5	24.2	18.8	16.0	13.9	12%	16%	24%
Narayana Hrudayalaya	787	1,60,995	44,121	49,088	53,850	9,286	10,503	11,563	5,669	5,946	6,781	27.8	29.1	32.3	28.3	27.1	24.4	17.8	15.7	14.3	10%	12%	8%
KIMS	1,375	1,10,110	22,421	26,186	30,036	6,150	7,355	8,300	3,278	3,895	4,423	40.7	48.2	55.7	33.8	28.5	24.7	18.6	15.5	13.8	16%	16%	17%
Global Health	494	1,32,657	26,813	30,541	35,303	6,081	6,973	8,092	3,001	3,732	4,510	11.4	14.0	16.8	120.6	98.6	81.8	22.3	19.4	16.7	15%	15%	21%
Shalby	124	13,441	8,110	9,406	10,151	1,558	1,906	2,048	749	1,012	1,183	7.8	10.1	11.0	175.8	136.3	125.0	9.0	7.3	6.8	12%	15%	19%
HCG (DnB Est)	**270**	**37,533**	**16,808**	**18,598**	**20,269**	**3,023**	**3,501**	**3,871**	**355**	**716**	**918**	**2.6**	**5.2**	**6.6**	**105.7**	**52.4**	**40.9**	**13.0**	**11.1**	**9.8**	**10%**	**13%**	**61%**
V/s BB Consesus	270	37,533	16,858	18,597	20,566	3,023	3,619	4,119	374	982	1,413	2.8	7.2	10.3	498.2	192.0	133.5	14.8	12.4	10.9	10%	17%	93%
Difference			***-50***	***1***	***-297***	***-0***	***-118***	***-248***	***-19***	***-265***	***-495***	***-0.2***	***-2.0***	***-3.7***	***-392***	***-140***	***-93***				***-1%***	***-4%***	***-32%***

**Bloomberg Consensus for other hospitals*

Key Risk in the business

- Delayed payment from government schemes affecting working capital
- Dependency on single speciality & inability to scale
- Change of management/investors/professional management
- Slower-than-expected pace of break-even of new hospitals
- Slowdown in revenue intensity
- Execution risk in expansion projects
- Price capping by regulators
- Possibility of conflicts or disputes with existing doctor partners

Brief details about Healthcare Global Enterprises (HCG)

About Healthcare Global Enterprises (HCG)

- Healthcare Global Enterprises was founded in 1989 and operated by the name of Bangalore Institute of Oncology (BIO), by an oncologist Dr. B.S Ajaikumar
- Healthcare Global Enterprises (HCG), India's largest provider of cancer care, is at the forefront of the battle against cancer with 22 comprehensive cancer care centers and 400+ oncologists.
- Healthcare global generates revenue through the sale of oncological diagnostic services, oncological radiation services and oncological medicine and surgery.
- Healthcare Global Enterprises (HCG) is a pioneer in bringing the most advanced technology for cancer care to India. It was the first to bring technologies such as cyclotron, PET-CT scanners, IGRT (Image Guided Radiation Technology), cyberknife, FFF (flattening filter free) LINAC's, Tomotherapy-H and Radixact.

Courtesy to cancer patients

We are proud to cover an organization like HCG who has helped thousands of cancer patients fight this dangerous disease. We wholeheartedly stand by and support all cancer patients and their families. We do not intend to hurt them in any way and sincerely apologize if this report may have unintentionally hurt them in any way.

FAQs

HCG & Cancer Related FAQs	
1.	**What causes cancer?**
	Cancer is a genetic disease caused by changes to genes that control the way our cells function, especially how they grow and divide. Cancer develops due to genetic changes that can happen because of errors that occur as cells divide, damage to DNA caused by harmful substances in the environment (chemicals in tobacco smoke, UV rays) or if they were inherited from the parents
2.	**What are the various types of cancer?**
	There are over 200 types of cancer and these can be classified according to where they start in the body. The most common types of cancer include cancers of the head and neck, gastrointestinal, lung, breast, cervix uteri and prostate.
3.	**What are 4 stages of cancer?**
	Stage I is early stage where a small invasive mass or tumor is found. Stage II is where it spreads to nearby tissue (localized). Stage III is where it affects more surrounding tissue (regional). Stage IV is when it spreads to other tissues or organs beyond where it originated (distant).
4.	**What is the difference between malignant and benign tumor?**
	Benign (non-cancerous) tumors tend to grow slowly and do not spread. Malignant (cancerous) tumors can grow rapidly, invade, and destroy nearby normal tissues, and spread throughout the body.
5.	**What are the various tests used in the detection of cancer?**
	The various tests used in the detection of cancer are mammogram screening, ultrasound, MRI, biopsy, and PET-CT scan.
6.	**What is a PET Scanner and how is it used in treatment of cancer? What is the cost?**
	A Positron Emission Tomography (PET) scanner is a diagnostic device which is used to detect cancer and see how far it has spread. It uses a radioactive drug (tracer) to detect normal and abnormal metabolic activity. Cancer cells show up as bright spots on PET scans because they have a higher metabolic rate than do normal cells. The cost ranges between $1.7 million and $2.5 million.
7.	**What is a LINAC and how is it used in the treatment of cancer? What is the cost?**
	A linear accelerator is a machine used in radiation treatment. It uses microwave technology to accelerate electrons which exit the machine as high energy x-ray beams. The beams destroy cancer cells without affecting surrounding tissues. Estimated cost is $7.8 million but varies based on the type of LINAC.
8.	**How are robots used in the treatment of cancer? What is the cost?**
	Oncological surgeons are trained to use robotic platforms and the control the surgery. These aid in providing making the surgery less invasive. The robots tiny wristed arm can bend more efficiently than human wrists making it possible to reach hard-to-reach areas. Estimated cost is around $2 million.
9.	**What is the LINAC utilization rate at HCG?**
	The LINAC utilization rate in HCG is approximately 68% as of H1FY23. It increased from approximately 58%, the same period last year.
10.	**Why should one invest in the hospital sector? Why HCG?**
	India's healthcare sector is a fast-growing market which has grown at a rate of 22% CAGR since 2016. The growth of the Indian hospital sector is being driven by value-based healthcare and has a large scope for growth. HCG is the market leader in oncology, the fastest growing specialty. Moreover, its genomic research, technology, value-based healthcare, and asset-light business model are expected to drive revenue and profitability going forward. We believe that HCG's superior research and technology would improve its clinical outcomes which would drive volumes going forward thereby increasing the revenues. Asset light business model is expected to provide HCG reduced capital expenditure reducing CAPEX and depreciation and increasing profitability.
11.	**What are the various constraints in HCG's business? Are beds a constraint?**
	Oncology is an outpatient focused practice. Therefore, beds are not a constraint unlike other multi-speciality hospitals, unless in case of surgical oncology. The various constraints include clinical talent, capital, number of man-hours, machines, and know-how.
12.	**What is the difference between single specialty and multispecialty hospitals?**

Single specialty hospitals exclusively engage in the care and treatment of the patients suffering from a specific illness (e.g.: cardiovascular, oncological). Multispecialty hospitals are medical treatment facilities that offers specialized treatment for various medical conditions under a single roof.

13. What is genomics and how is it changing the world? What is the average cost of genomics?

Genomics is a branch of molecular biology concerned with the structure, function, evolution, and mapping of genomes (entire sequences of genes). Genomics are used in diagnosis and treatment of cancer, rare genetic diseases, choosing the right medication (pharmacogenomics), genome editing and others. Whole genome sequencing costs approximately Rs. 25,000 ($338 USD) to Rs. 50,000 ($676 USD) in India.

14. Which is the fastest growing specialty at 12% p.a.?

Oncology is the fastest growing specialty at 12% p.a

15. Does insurance cover cancer and what kind?

Private Healthcare Insurance:

Type Of Insurance Policy	Cancer Coverage
General Health Insurance	Most general health insurance policies cover all types of cancer treatments including hospitalization,diagnostics & medicines.Barely,3-5% of cost are not covered by general insurance policies
Critical Illness Insurance	It is cheaper than general healthcare insurance but more limited in scope since it only covers critical illnesses. It only covers cancer at advanced stages. For instance, it only covers cancer if the malignant tumor shows uncontrolled growth with invasion and destruction of tissues
Cancer Insurance	A cancer specific insurance policy covers a number of costs associated with cancer diagnosis and treatment, including hospitalization, radiotherapy, chemotherapy, surgery and others. Most cancer insurance policies do not cover skin cancer. Existing cancer insurance policies in the market include Cancer Care Plus by ICICI Prudential, HDFC Life Cancer, AEGON Religare has just launched its iCancer Insurance Plan.

Government Healthcare Insurance:

Type Of Insurance Policy	Cancer Coverage
Ayushman Bharat Yojana Scheme	Ayushman Bharat is one of the largest healthcare schemes in the world, offering health insurance of up to INR 5 lakh. It includes coverage of only prostate cancer. It does not include other types of cancer.

16. Are there any price-cap regulations on oncology services in India?

No, there are no price-cap regulations on oncology services in India. Oncology service providers can decide their own pricing.

17. What is national cancer registry?

National cancer registry conducts systematic collection of data on cancer patients, performed by its various population-based cancer registries (PBCRs) and hospital-based cancer registries (HBCRs) across India.

18. Can India become a hub for cancer care?

India is well positioned to become a hub for cancer care. India offers the best value for money and quality care for cancer patients as compared to its global peers. Indian cancer care providers are also able to treat more cancer patients without expanding bed capacity through tele-health and their unique business models (e.g.: hub and spoke).

Gaining traction of India on global forum will also be a major driver for foreign patients in India who are seeking affordable quality healthcare.

Financials

P&L (Rs mn)	FY20	FY21	FY22	FY23E	FY24E	FY25E
Net Sales	10,956	10,134	13,978	16,808	18,598	20,269
Operating Expenses	-2,399	-2,403	-3,549	-4,152	-4,650	-5,067
Employee Cost	-2,080	-1,959	-2,337	-2,750	-3,031	-3,284
Other Expenses	-4,756	-4,506	-5,713	-6,884	-7,416	-8,047
Operating Profit	**1,722**	**1,266**	**2,380**	**3,023**	**3,501**	**3,871**
Depreciation	-1,485	-1,592	-1,583	-1,658	-1,744	-1,885
PBIT	**237**	**-326**	**797**	**1,365**	**1,757**	**1,986**
Other income	70	170	127	-	-	-
Interest	-1,377	-1,192	-978	-1,019	-978	-938
PBT	-1,069	-1,349	-54	346	779	1,049
Profit before tax (post exceptional)	-1,069	-2,283	892	346	779	1,049
Provision for tax	-62	76	-489	-124	-196	-264
Profit & Loss from Associates/JV	-123	-4	-14	-14	-14	-14
Reported PAT	**-1,255**	**-2,211**	**389**	**207**	**568**	**770**
MI	188	276	148	148	148	148
Owners PAT	**-1,067**	**-1,935**	**537**	**355**	**716**	**918**
Adjusted Profit (excl Exceptionals)	**-1,067**	**-1,144**	**-53**	**355**	**716**	**918**

Balance Sheet (Rs mn)	FY20	FY21	FY22	FY23E	FY24E	FY25E
Equity capital	887	1,254	1,390	1,390	1,390	1,390
Reserves	2,926	5,718	7,313	7,668	8,384	9,302
Net worth	**3,813**	**6,972**	**8,703**	**9,058**	**9,774**	**10,692**
MI	385	168	134	134	134	134
Non Current Liabilites	12,576	8,564	8,661	8,166	7,671	7,177
Current Liabilites	5,758	4,653	4,698	4,655	4,654	4,688
TOTAL LIABILITIES	**22,532**	**20,356**	**22,195**	**22,013**	**22,234**	**22,692**
Non Current Assets	19,494	16,006	17,172	16,338	15,420	14,863
Fixed Assets	11,145	10,010	11,643	10,768	9,807	9,205
Right of Use Assets	5,776	4,114	4,045	4,045	4,045	4,045
Financial Assets	563	431	634	634	634	634
Deferred Tax Asset	261	343	60	61	63	65
Long Term Loans and Advances	516	451	-	-	-	-
Other Non Current Assets	1,232	658	790	829	871	914
Current Assets	**3,038**	**4,350**	**5,023**	**5,675**	**6,814**	**7,829**
Current investments	-	-	-	-	-	-
Inventories	233	211	300	339	380	414
Trade Receivables	1,857	1,866	2,175	2,523	2,791	3,042
Cash and Bank Balances	320	408	1,975	2,237	3,066	3,793
Short Term Loans and Advances	54	93	16	18	19	21
Other Financial Assets	275	1,546	341	341	341	341
Other Current Assets	300	225	217	217	217	217
TOTAL ASSETS	**22,532**	**20,356**	**22,195**	**22,013**	**22,234**	**22,692**

Cashflow (Rs mn)	FY20	FY21	FY22	FY23E	FY24E	FY25E
PBT	-1,069	-2,283	892	346	779	1,049
Depreciation	1,485	1,592	1,583	1,658	1,744	1,885
Net Chg in WC	-109	-586	87	-47	-36	-55
Taxes	-267	380	-237	-124	-196	-264
Others	1,262	2,102	-124	702	744	760
CFO	**1,301**	**1,205**	**2,201**	**2,534**	**3,035**	**3,374**
Capex	-1,074	-354	-704	-1,000	-1,000	-1,500
Net Investments made	56	-1,429	1,316	-	-	-
Others	4	71	-1,857	-	-	-
CFI	**-1,014**	**-1,711**	**-1,246**	**-1,000**	**-1,000**	**-1,500**
Change in Share capital	203	4,962	1,322	-	-	-
Change in Debts	231	-2,723	-1,938	-603	-577	-558
Div. & Div Tax	-	-	-	-	-	-
Others	-610	-1,645	1,227	-670	-629	-588
CFF	**-176**	**594**	**611**	**-1,273**	**-1,206**	**-1,146**
Total Cash Generated	**112**	**88**	**1,567**	**262**	**829**	**728**
Cash Opening Balance	**209**	**320**	**408**	**1,975**	**2,237**	**3,066**
Cash Closing Balance	**320**	**408**	**1,975**	**2,237**	**3,066**	**3,793**

Ratios	FY20	FY21	FY22	FY23E	FY24E	FY25E
OPM	15.7	12.5	17.0	18.0	18.8	19.1
NPM	-9.7	-11.1	-0.4	2.1	3.9	4.5
Tax rate	5.8	-3.3	-54.8	-36.0	-25.2	-25.2
Growth Ratios (%)						
Net Sales	11.9	-7.5	37.9	20.2	10.7	9.0
Operating Profit	37.5	-26.5	88.0	27.0	15.8	10.6
PBIT	-35.4	-237.5	-344.2	71.2	28.8	13.0
PAT	306.2	76.2	-117.6	-46.8	174.7	35.5
Per Share (Rs.)						
Net Earnings (EPS)	-12.03	-15.43	3.87	2.55	5.15	6.60
Cash Earnings (CPS)	4.71	-2.73	15.25	14.48	17.70	20.17
Dividend	-	-	-	-	-	-
Book Value	42.99	55.61	62.61	65.16	70.31	76.92
Free Cash Flow	18.34	5.51	14.61	10.68	14.55	13.06
Valuation Ratios						
P/E(x)	-22	-17	70	106	52	41
P/B(x)	6	5	4	4	4	4
EV/EBIDTA(x)	17	29	16	13	11	10
Div. Yield(%)	-	-	-	-	-	-
FCF Yield(%)	6.79	2.04	5.41	3.95	5.39	4.84
Return Ratios (%)						
ROE	-28%	-16%	-1%	4%	7%	9%
ROCE	3%	-1%	7%	10%	13%	13%
RoIC	3%	-1%	4%	8%	12%	14%

Source: Company, Dalal & Broacha Research

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